



HIGHER HEALTH

Higher Education & Training: Health, Wellness and Development Centre

ANNUAL REPORT

2019
2020

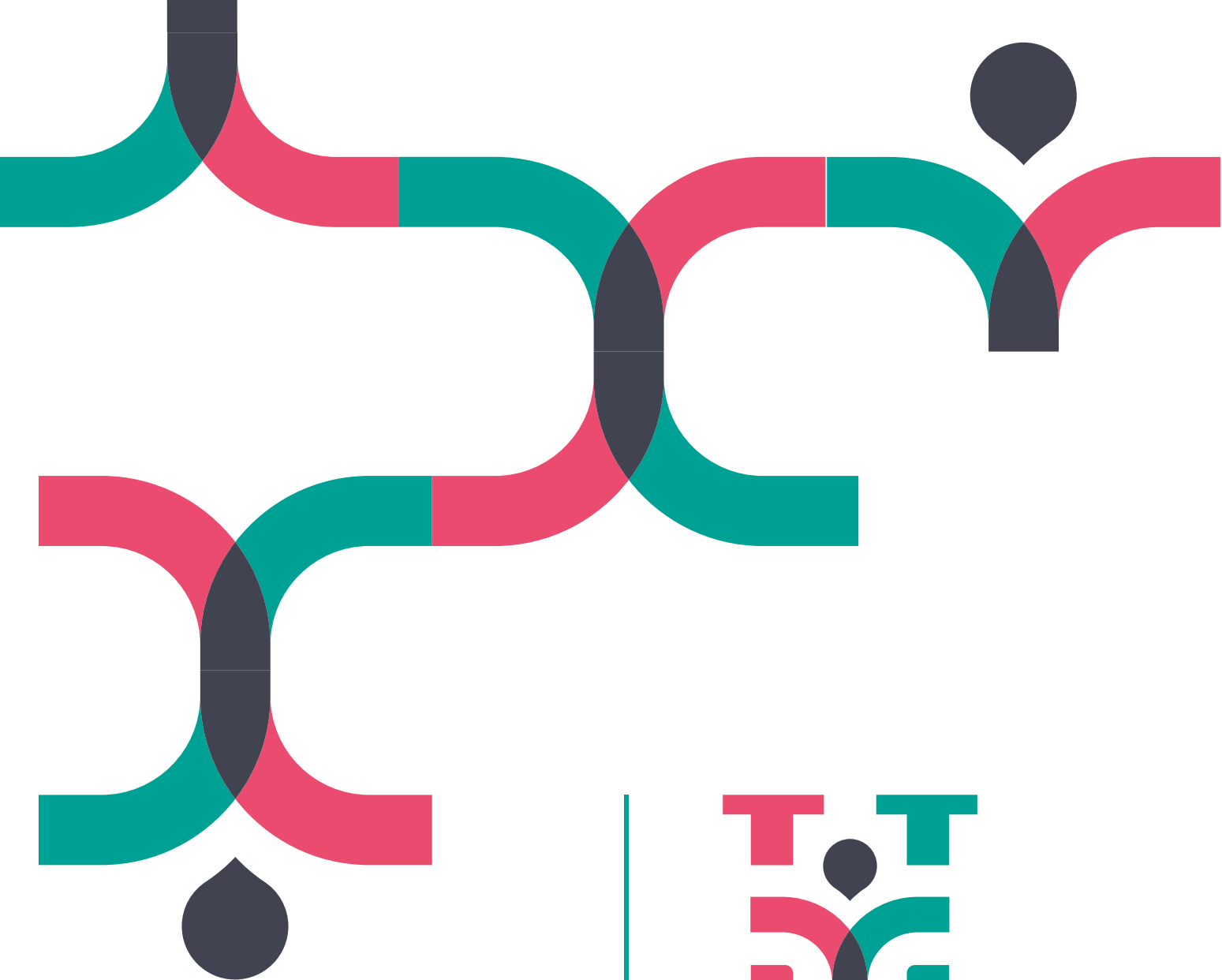


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CHAIRPERSON'S MESSAGE

As the 2019/20 reporting year drew to a close, the COVID-19 pandemic had just begun to make its destructive power felt in South Africa. All our university and TVET college campuses were closed, our students were sheltering at home and we were all suddenly conscious – as never before – of the immense value of good health to each of us personally, to our communities and our nation.

The crisis also revealed the true worth HIGHER HEALTH's ongoing investment in developing a robust, holistic primary healthcare programme across our sector and mobilising a range of role-players to help realise this vision. A crisis is no time to establish relationships and set up structures – it is preferable by far to be able to call on established partners and working systems. HIGHER HEALTH demonstrated its ability to do so as it rallied to support the Department of Higher Education and Training in planning a safe return of students and staff to campuses across the land.

HIGHER HEALTH has positioned itself uniquely at the intersection of health and wellness, on the one hand, and higher education, on the other. The experience it has gained during two decades of straddling these two worlds has enabled the organisation to play a strategic role in building bridges and advising higher education decision-makers at national and institutional level.

Over the years, our organisation has argued powerfully for recognition of the extent to which health and wellbeing impact on the ability of students to study effectively, succeed academically and join the ranks of graduates. The return on investment in student health and wellbeing has the potential to accrue at many levels.

PROF WIM DE VILLIERS
CHAIRPERSON



At the level of the individual student, appropriate health interventions – particularly in fields such as mental health, sexual and reproductive health and HIV prevention – may have lifelong benefits. The period immediately after leaving school brings great opportunity and great stress to students as they find their place in an adult world and a competitive academic setting. Our approach simply cannot be “survival of the fittest”. We owe it to our students to provide a learning environment in which they can flourish – and in many cases make the leap into a better life than their parents experienced.

At the level of universities and colleges, the improved health and emotional stability of the student population has the potential to help improve throughput rates and increase the supply of graduates to a skills-hungry labour market. In addition, employers have the prospect of hiring young graduates who have not only gained academic and technical knowledge but have had a chance to acquire many of the “soft skills” required for a successful working life.

Government and the South African tax payer invest enormous sums in higher education and training. In recent years this amount has increased with larger allocations to the National Student Financial Aid Scheme (NSFAS), as government has attempted to respond to the extreme financial distress of many students. By complementing this financial support with psychological support and accessible healthcare, HIGHER HEALTH enhances the likelihood of public funds being fruitfully invested in higher education and training.

The concept of the Second Curriculum is at the heart of building student competency in relation to health and wellness. This refers to a range of learning opportunities outside of the accredited curriculum through which students gain knowledge and personal skills to live more healthily. These activities have grown steadily over the years, and 2019 was a watershed year in terms of shaping this approach into a more structured programme with a custom-built curriculum, well-trained facilitators and

firm targets for output. It rests on the huge expansion of HIGHER HEALTH's staffing structure to ensure that every university and TVET college has at least one campus health and wellness coordinator (CHWC). The incredible value of having trained and committed “boots on the ground” has been evident in our ability to help campuses deal with the COVID-19 epidemic.

We are profoundly grateful to our funders, without whom such an ambitious expansion would have been impossible, and to higher education and training institutions that have embraced new ways of working. Every aspect of the HIGHER HEALTH enterprise rests on partnerships and collaboration.

When we consider the road that we have walked, together with our partners, towards comprehensive primary healthcare at campus level, we can see considerable progress. We have revolutionised our relationship with the public health sector and been integrated into district health operations. We have been able to facilitate access to relevant and convenient on-campus health services for hundreds of thousands of students. We have provided platforms for students to develop leadership skills while addressing the health and wellness agenda. We have been alive to the health issues that students themselves are concerned about, such as gender-based violence and the psychological stress experienced by a high proportion of students, and driven a response to these problems.

HIGHER HEALTH has been strategic in diversifying its sources of funding, seeking out synergies with diverse organisations and pursuing multiple collaborations. This has gone a long way to enabling our expansion and ensuring sustainability. Perhaps the underlying attraction of HIGHER HEALTH is that it offers an opportunity to invest in the future of our young people. Many organisations and individuals are strongly drawn to this mission and ready to join forces – which is why we have been able to create a programme that is much bigger and stronger than the sum of its parts.

DEPUTY CHAIRPERSON'S MESSAGE

When HIGHER HEALTH included technical and vocational education and training (TVET) colleges in its programme several years ago, it was motivated largely by the idea of levelling the playing field for young people at TVET colleges and universities, respectively, in terms of their access to health and wellness programmes.

A hallmark of the organisation's work in the TVET sector has been its institutional capacity building, enabling management, staff and students of colleges to engage with the challenges of creating a campus health programme that not only enhances students' wellness but also assists their performance in their course of study.

One of the most empowering contributions of HIGHER HEALTH has been the research it has conducted in the sub-sector, for example, with regard to the capacity of colleges to include health in their formal teaching, the knowledge and attitudes of students and staff to HIV and sexual health, and the impact of GBV and mental health on students and staff. The findings of these studies have enabled us to reflect on our situation and devise ways to improve it.

HIGHER HEALTH's supply of conceptual tools – policies, guidelines and training materials – as well as the convening of regional forums of colleges and the building of stakeholder networks have had a profoundly enabling impact on colleges. The most tangible result of these partnerships has been the establishment of many campus clinics by provincial and district health authorities.



MR SANELE MLOTSHWA

DEPUTY CHAIRPERSON

Less tangible, but equally important, has been the spirit of inclusiveness that has characterised HIGHER HEALTH's approach. Its activities have brought TVET colleges and universities closer together in some regions, focusing them on shared goals and outcomes. These are precious opportunities in a sector where universities and colleges have often functioned quite separately. We look forward to a time when private colleges and community colleges will also be encompassed in the HIGHER HEALTH vision.

It is to HIGHER HEALTH's credit that it has been prepared to employ asymmetrical approaches to supporting universities and colleges respectively, in order to obtain equitable outcomes. TVET colleges are usually quite lean institutions and do not always have the student support services that many universities can rely on.

We certainly welcome HIGHER HEALTH's deployment of a campus health and wellness coordinator (CHWC) to every college. We are also encouraged that TVET colleges were selected for the implementation of the adolescent girls and young women's programme funded by the

Global Fund. This is a further step towards equity, as students from particularly disadvantaged environments will have a unique opportunity to join a structured multi-year health programme.

When the COVID-19 pandemic forced the closure of all educational institutions in March and we had to rethink fundamentally what health and safety meant, TVET colleges found themselves reaching for the lessons they had learnt while working in partnership with HIGHER HEALTH. The value of clear policies and guidelines, capacity building processes and communication systems had been well learnt. The network of CHCWs, peer mentors and peer educators became a precious resource that could be repurposed in this crisis.

In short, development has been at the heart of the relationship between HIGHER HEALTH and TVET colleges. The pace of progress has varied and not every experiment has borne fruit. But overall a valuable new dimension has been added to the student experience at our colleges in a sustainable manner that can only grow in the future, given access to appropriate human and financial resources.

01

SECTION 1

OVERVIEW OF THE CHIEF EXECUTIVE OFFICER

In 2019, the Higher Education and Training Health, Wellness and Development Centre exchanged its familiar and trusted HEAIDS brand for a fresh organisational identity: HIGHER HEALTH. The new brand speaks unambiguously to our strategic vision of improving the academic achievement of students in the higher education and training sector by enhancing their health, wellness and psychosocial wellbeing.

DR RAMNEEK AHLUWALIA
CHIEF EXECUTIVE OFFICER

The adoption of the HIGHER HEALTH brand gave a hint to the outside world of the huge transformation occurring inside HIGHER HEALTH and the impact this would have on health and social development programmes at the **26 public universities** and **50 technical and vocational education and training (TVET) colleges** that are partners in the HIGHER HEALTH journey. These programmes have been shaped as a response to pervasive challenges confronting students, including gender-based violence, psychological stress, substance use and maintaining sexual health.



A snapshot of various aspects of the change process is presented below, and each element is described in more detail later in this overview.

STRATEGIC DIRECTION

A corporate strategy for HIGHER HEALTH was developed, focusing on the organisation's long-term role and blending the strategic contribution of health programmes to the higher education sector with matters of organisational governance and sustainability.

BUDGET EXPANSION

HIGHER HEALTH has been extremely successful in mobilising financial resources and diversifying its funding sources. Our income doubled in the reporting year and this made it possible to expand our support to institutions and strengthen our management capacity.

PROGRAMME DELIVERY MODEL

Some years ago, HIGHER HEALTH developed a model for the delivery of a holistic health and psychosocial support programme to students at universities and TVET colleges. In 2019, decisive progress was made towards the full realisation of this model in practice. The main features of this model have remained consistent over the years (see Figure 1 on page 9), but some important modifications have been made.

STRATEGIC CONTRIBUTION TO THE SECTOR

HIGHER HEALTH is recognised as a centre of excellence for the development of comprehensive primary healthcare in higher education settings. As such, it has become a valued adviser to the Department of Higher Education and Training (DHET) and authorities in other countries.

STRENGTHENING CAPACITY TO SUPPORT INSTITUTIONS

- Decentralised management: Three regional HIGHER HEALTH offices and six provincial offices have been established, bringing our managers much closer to the institutions we serve. As our investment in campus-based health initiatives increases, our responsibilities grow in terms of working with institutions and other partners to derive the best possible benefit from resources.
- Stronger operational capacity at institutions: In another first, HIGHER HEALTH appointed 62 campus health and wellness coordinators (CHWCs) who are based at every TVET college and

two universities. The organisation also began to appoint and provide stipends to 250 mentors in the peer education programme at TVET colleges.

ENHANCED DELIVERY OF THE SECOND CURRICULUM

The construction of this nationwide network of CHWCs and mentors has been the key to delivering the Second Curriculum consistently and at scale. The Second Curriculum comprises a range of extramural health-related learning opportunities that enable students to assume responsibility for their physical and emotional health. In 2019/20, HIGHER HEALTH built a robust platform for implementing the Second Curriculum that takes account of different realities at universities and TVET colleges.

SCREENING FOR HEALTH RISK AND TARGETED ASSISTANCE

HIGHER HEALTH has developed a self-assessment tool for students that is being used within the Second Curriculum to help individuals understand whether they may be at risk in terms of their sexual health (including HIV), their mental health or gender-based violence. This serves two purposes – prioritising students for counselling and other health services and motivating them to adapt their personal lifestyles, where needed.

INNOVATION IN MEETING WOMAN STUDENTS' NEEDS

Woman students at selected TVET colleges will have the opportunity of enrolling in a comprehensive health programme that will respond to their needs and track their health status until they complete their studies. Part of the service is free access to a range of sexual health and personal hygiene products from sponsored vending machines placed on campus. This pilot project has been funded by the Global Fund through a grant to the Networking AIDS Consortium of Southern Africa (NACOSA).

While change was definitely the dominant theme of 2019/20, the established interventions of HIGHER HEALTH were sustained throughout this period, although on a smaller scale. These included First Things First health and wellness days – which combine health promotion and screening for various communicable and non-communicable diseases – and building capacity to offer students relevant clinic services right on campus. These, too, are described in more detail later in this report.

1.1

DEFINING OUR FUTURE DIRECTION

One of the consequences of HIGHER HEALTH establishing itself as an independent organisation in 2017 – after functioning fruitfully for many years as a programme under the umbrella of Universities South Africa (USAf) – was that it needed a new kind of strategy.

The strategic thinking of the past had not only been strongly centred on addressing HIV and TB among students, but it had been appropriate to a programme rather than a fully-fledged organisation. Recognising this, the Board of Directors initiated the development of a strategy to guide the organisation for the period 2020 to 2025.

Discussions focused largely on the proper focus and scope of health interventions within a sector where the essential function is post-school learning, teaching and research. The debates took place at a time when the issues of poverty and

vulnerability among students had already been highlighted by the #feesmustfall movement and the question of institutions' duty of care to their students was a matter for critical reflection. The balance achieved by the Board is reflected in the vision and goals set out on page 8. The goals also speak to HIGHER HEALTH's corporate ethos and concerns: good custodianship of funding, recognition of partnerships as the lifeblood of the organisation, and the dependence of the entire enterprise on successful mobilisation of funding.

HIGHER HEALTH is increasingly pursuing its mission by playing a strategic advisory function on health and wellness matters across the post-school education and training sector.

The strategy lists numerous objectives and just a few are mentioned here, relating specifically to health programmes. They included the prevention of ill-health among students, ensuring students have access to quality healthcare and psychosocial services, the development of health awareness and promotion of healthy lifestyles among students, and addressing social and cultural aspects of campus life that impact on student health, wellness and development.

HIGHER HEALTH'S VISION AND GOALS

VISION



To be a primary instrument in our post-school education and training system for improving student success rates and study completion by enhancing their overall health and psychosocial wellbeing.

GOALS



- Enhance student health and wellness in order to increase the proportion of students who complete their studies and reduce the time it takes for students to graduate.
- Build the knowledge, understanding, skills and capacity of students in relation to general health and holistic wellness. Help produce graduates who are well-prepared for family and career responsibilities by developing their grasp of the importance of health in these contexts.
- Develop a campus environment that is conducive to health by nurturing a commitment to human rights as a fundamental aspect of student development and wellbeing.

- Improving organisational effectiveness and efficiency in order to reduce organisational costs and increase funding for health and wellness-related programmes.
- Strengthen and expand partnerships with higher education institutions, relevant government departments, funding and donor agencies and other organisations active in our field.
- Strengthen and diversify our funding base, ensuring a variety of funding sources in order to secure operational sustainability.

FOUNDATIONS

- Sustainable Development Goals
- National Development Plan 2030
- White Paper of the Department of Higher Education and Training
- National Strategic Plan for HIV, TB and STIs 2017–2022

1.2

MOBILISATION OF FUNDS

HIGHER HEALTH recognises the importance of diversifying its funding sources so that the loss of any single funder is not a major threat to the sustainability of the organisation and its activities. Because of our comprehensive approach to health and wellness, we interact with a range of international and national organisations, many of which are prospective funding partners.

We proactively identify programme priorities that we share with these organisations and seek specific funding for these aspects of our work. While this requires long-range planning and careful proposal preparation, the approach has clearly paid dividends in terms of increasing our overall funding pool and the stability of the organisation.

1.3

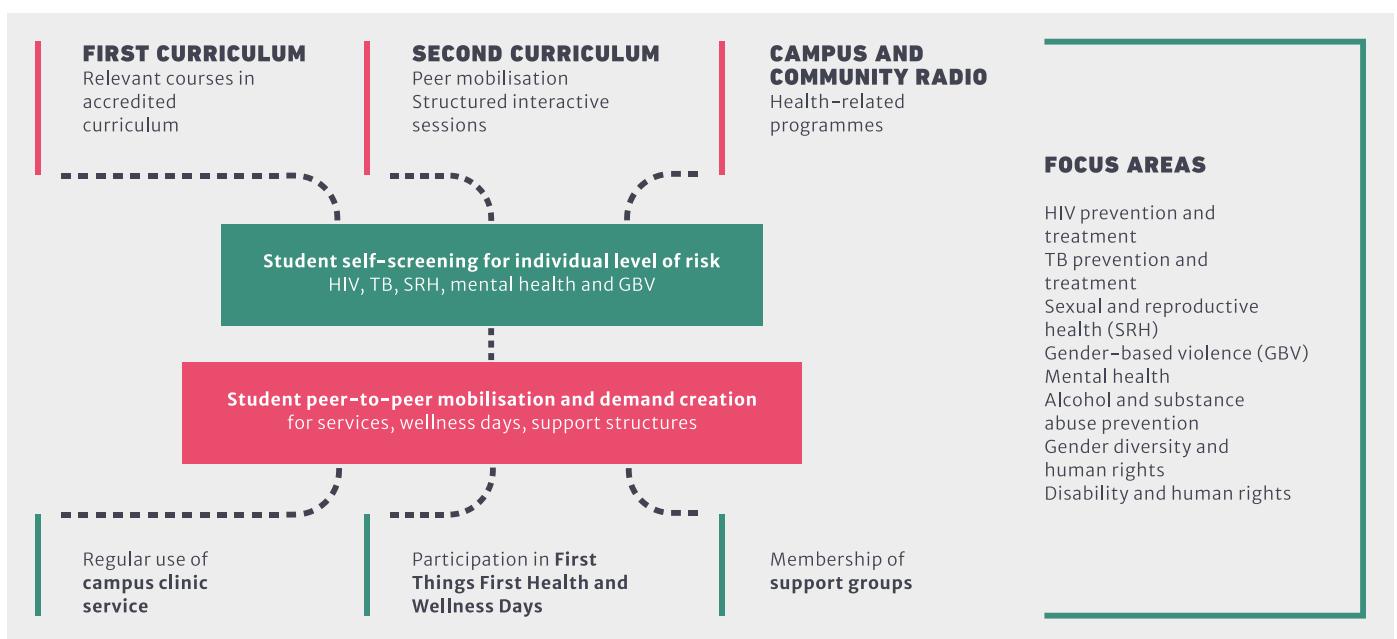
ROBUST PROGRAMME MODEL

Over several years, the model for delivering health programmes to some **420 campuses** across the country has evolved. It started with a simple cluster of focus areas and the notion that mobilisation of students to take control of their own wellbeing was essential.

Gradually our ideas about the peer-to-peer education system and the Second Curriculum evolved, together with the ambition to ensure students had better access to health services that answered their most pressing needs.

The most recent refinement of the model is presented in Figure 1. It depicts both the action systems and the focus areas.

Figure 1: HIGHER HEALTH model for delivering a comprehensive health and wellness programme



1.4

INCREASING SUPPORT FOR INSTITUTIONS

HIGHER HEALTH is essentially an enabler and a catalyst for health action in the higher education and training sector. The programme of services and activities outlined above is delivered mainly by universities and colleges in partnership with local healthcare providers.

However, as the programme has expanded, there has been a need to increase HIGHER HEALTH's support to institutions. TVET colleges, in particular, often lack the human resources to undertake this range of interventions in addition to their primary learning and teaching function.

In 2019/20, HIGHER HEALTH moved to address this need in three ways:

- We appointed campus health and wellness coordinators (CHWCs) at every TVET college and two universities, Walter Sisulu University and Mangosuthu University of Technology. There are 62 CHWCs in all and they are usually based at the largest campus of each institution. They are all employed directly by HIGHER HEALTH and their sole responsibility is to deliver the organisation's programme in close collaboration with the institution.
- Many universities have dedicated staff to deal with student health, wellness and related matters – such as gender – and HIGHER HEALTH's direct grant to universities may be used to enhance existing human resource capacity.

- We decentralised our management functions by establishing three regional offices – in Centurion, Durban and Cape Town – and six provincial offices, often situated in provinces where there is no regional office. The configuration of this regional system is set out in Figure 2.
- The role of managers in these offices is partly to oversee and support the CHWCs within their areas of operation. It is also to monitor the output the sector is achieving through this considerable investment and the impact in terms of benefits to students. Equally importantly, regional and provincial managers will be much more accessible to institutions and other local programme partners and able to drive collaboration more strongly – and sensitively – than is possible over vast distances.
- We also strengthened our head office management team to ensure effective communication and coordination across the country and the performance of core management functions to ensure the sustainability of HIGHER HEALTH. The positions of Director of Programmes and Executive Manager in the CEO's office were created and filled.

Our employment numbers sum up the extent of the change in HIGHER HEALTH. At the end of the last financial year, we had **31 employees** and by the end of 2019 the figure was over **90**. The necessary office space was secured, a fleet of 10 branded vehicles purchased, and an IT network developed to ensure smooth and secure communication among offices.

Figure 2: HIGHER HEALTH capacity at regional, provincial and institutional level

Northern Region	Eastern Region	Cape Region
Gauteng, Limpopo and North West	Free State, KwaZulu-Natal and Mpumalanga	Eastern Cape, Northern Cape and Western Cape
Regional office: Centurion Regional manager Two provincial monitoring and evaluation coordinators	Regional office: Durban Regional manager One provincial monitoring and evaluation coordinator	Regional office: Cape Town Regional manager Two provincial monitoring and evaluation coordinators
Provincial office: Polokwane One provincial monitoring and evaluation coordinator	Provincial offices: Bloemfontein, Newcastle and Nelspruit One provincial monitoring and evaluation coordinator per office	Provincial offices: Mthata and Port Elizabeth One provincial monitoring and evaluation coordinator per office
A total of 19 CHWCs based on campuses Gauteng: 8 Limpopo: 8 North West: 3	A total of 23 CHWCs based on campuses Free State: 6 KwaZulu-Natal: 11 Mpumalanga: 6	A total of 20 CHWCs based on campuses Eastern Cape: 12 Northern Cape: 2 Western Cape: 5

1.5

INTENSIFYING SECOND CURRICULUM ENGAGEMENT

The Second Curriculum refers to a range of extramural learning opportunities provided on campus through which students learn about health and wellness and develop the personal skills to take charge of their wellbeing.

- It is open to students in all years of study and across all disciplines. Participation of students is entirely voluntary.
- The core content of the Second Curriculum is now contained in a purpose-built curriculum document which ensures a consistent approach across the country. Modules correspond to health and psycho-social matters that are critically important to young people.
- The style of teaching and learning is less formal than in the academic curriculum and highly reflective and interactive.

The concept of the Second Curriculum has evolved over the years. Initially it was delivered through student participation in health activations and campus dialogues and their exposure to radio programmes. While these remain valuable elements of the Second Curriculum, a more structured approach to delivering the content has also been introduced. It centres on one-hour learning sessions for different curriculum modules that are presented and guided by a skilled facilitator.

Thanks to the deployment of CHWCs across the system and provision of stipends to peer mentors, it will be possible to implement this key addition to the Second Curriculum on a national scale. At universities, Second Curriculum sessions take place after lectures, while TVET colleges offer them during recess periods within college hours.

The learning sessions are facilitated by peer mentors and CHWCs and a maximum of 50 students participate in each session. Peer educators generate interest in the session and assist with facilitation.

There was an intensive period of training and preparation in late 2019 and the first sessions were introduced in January and February 2020. They covered:

- HIV, TB and STIs.
- SRH.
- Domestic, partner and gender-based violence.
- Mental health.

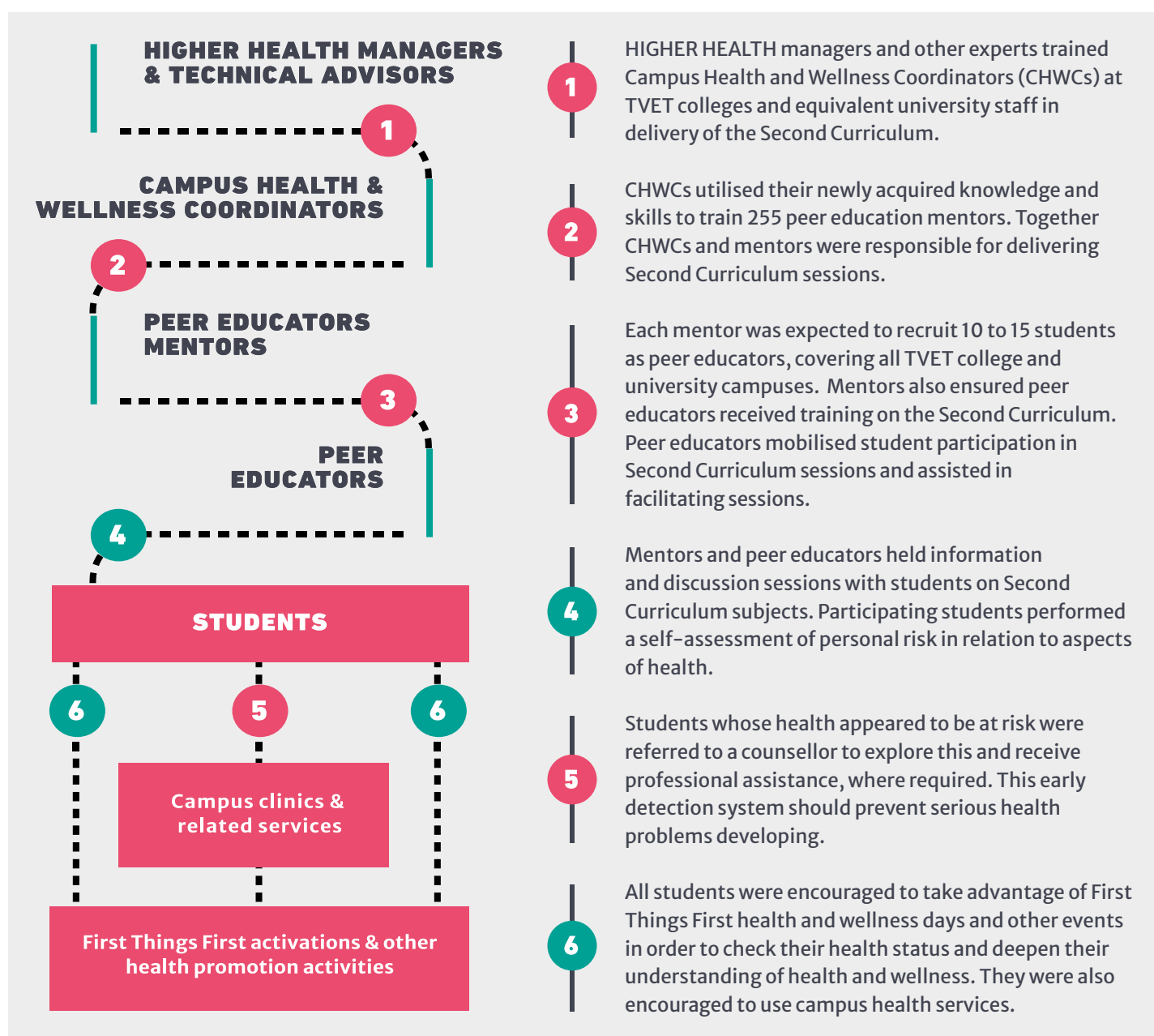
Total attendance across the modules was 64 717 during the initial three-month period, indicating that the system has the potential to deliver at scale when

sessions are sustained throughout the academic year. The eventual target is to accommodate 900 000 students a year in sessions covering different modules.

This new delivery system represents a massive investment of financial and human resources. Each institution has output targets to meet and performance will be managed against these.

It is important to emphasise that this programme does not unfold in a vacuum, but in the context of the health education and mass screenings of the First Things First Health and Wellness Programme, other peer-to-peer awareness-raising activities and the expansion of primary healthcare services on campuses.

Figure 3: Delivering the HIGHER HEALTH Second Curriculum



PEER-TO-PEER COMMUNICATION

The active participation of students in planning and executing HIGHER HEALTH activities is a hallmark of the programme. Each year, universities and colleges recruit a fresh intake of peer educators who volunteer to promote healthy living to their peers.

They are trained in basic facts related to the seven focus areas, sensitised to human rights dimensions, encouraged to tap into relevant personal experiences, and helped to develop communication skills. Peer educators organise

dialogues and group discussions, mobilise participation in health and wellness days, market campus health services, and undertake one-on-one communication. They also play a supporting role in facilitating Second Curriculum sessions.

Peer education mentors are the driving force behind the recruitment, training and support of peer educators. Typically, they are staff members of institutions, teaching life orientation or responsible for student support services. HIGHER HEALTH is at last in a position to provide a small stipend to 250 TVET college mentors in recognition of the valuable work they do. At universities, there are generally staff members whose established role encompasses the activities of peer mentors.

1.6

RISK ASSESSMENT FOR INCREASED EFFICACY

The introduction of a process for students to conduct self-screening for various health risks is innovative and by no means standard practice. HIGHER HEALTH believes that it may have several distinct benefits and will monitor closely to see if these are achieved.

The risk assessment is a standard self-administered questionnaire usually completed in the supportive environment of a Second Curriculum session dealing with the topic covered in the questionnaire. Students are given

information that enables them to rate their risk according to the answers they have given. Currently there are assessments for: HIV, STIs and TB, SRH, GBV and mental health. They are intended as a tool for disease prevention and early intervention.

In terms of prevention, the questions alert students to certain behavioural and environmental risks to health and assist them to consider what part these risks play in their lives. In theory, this may not only increase their risk

perception but also their motivation to take measures to limit their risk.

In terms of early detection, the assessment asks students whether they are experiencing signs and symptoms of various conditions – for example, TB, STIs and common mental health problems – and their answers facilitate referral for care, where needed. The intention is to secure professional assistance at an early stage when most conditions are easier to treat.

In January to March 2020, a total of 108 929 self-assessments were completed, the greatest number in relation to HIV, TB and STIs. The intervention was adopted more strongly in this initial period by TVET colleges than universities.



1.7

SPOTLIGHT ON YOUNG WOMEN

The particular vulnerability of young women to HIV infection is a well-established fact. The roots of this are complex and relate largely to social, economic and cultural features of our patriarchal society. Recently programmes such as the PEPFAR-sponsored DREAMS initiative have recognised that simple, practical steps can contribute to the empowerment of young women and thereby reduce their risk of HIV infection. DREAMS aimed, among other things, to ensure young women had access to condoms and contraception, that they remained in school, and utilised available social security benefits.

Against this background, the Global Fund began to finance a programme for adolescent girls and young women (AGYW) at TVET colleges. While woman students have always been a priority for HIGHER HEALTH, this grant has enabled us to engage more intensely with a cohort of woman students at 10 TVET colleges.

Young women who enrol in the programme will be followed up regularly by our CHCWs throughout their

years of study. They are encouraged to test for HIV periodically and, if positive, linked to ART. The majority who are negative are counselled on using available HIV prevention methods, including condoms and pre-exposure prophylaxis (PrEP), as well as contraception and SRH services. They participate in Second Curriculum sessions and wellness days.

The colleges participating in the programme for adolescent girls and young women are among those that will be provided with vending machines from which students will be able to obtain – at no cost – male and female condoms and other barrier products, pregnancy tests and sanitary protection. These machines have the potential to address several obstacles young women encounter in acquiring sexual health products. The first barrier is cost; the second physical access; and the third the lack of privacy when accessing these items in various other settings.

The initiative is funded as a pilot project and its impact will be closely monitored.

Figure 4: Profile of the students we serve

HIGHER HEALTH supports 26 public universities and 50 TVET colleges with a combined enrolment of 1 742 701 students (2018 figures).

- At both universities and TVET colleges **woman students outnumber men.**
- A high proportion of university students utilise **distance learning** methods.
- At TVET colleges many students study **part-time.**
- University graduation rates, particularly among undergraduates, are worryingly low.
- Students at TVET colleges tend to be younger than those at universities.
- While students are mainly in their **late teens and early 20s**, a substantial minority are older.

PUBLIC UNIVERSITIES

Total 2018 enrolment 1 085 568
Growing steadily

♀ **59%** ♂ **41%**

Distance learning students **37%**
Contact learning students **63%**
Students with disabilities **0.8%**

Number graduating 227 188
Women **62%** Men **38%**

Graduation rates*

Postgraduate below Masters **47%**
Masters **23%**
Undergraduate certificate & diplomas **21%**
Undergraduate degrees **17.2%**
Doctoral degrees **14.1%**

TVET COLLEGES

Total 2018 enrolment 657 133
Declining since 2014

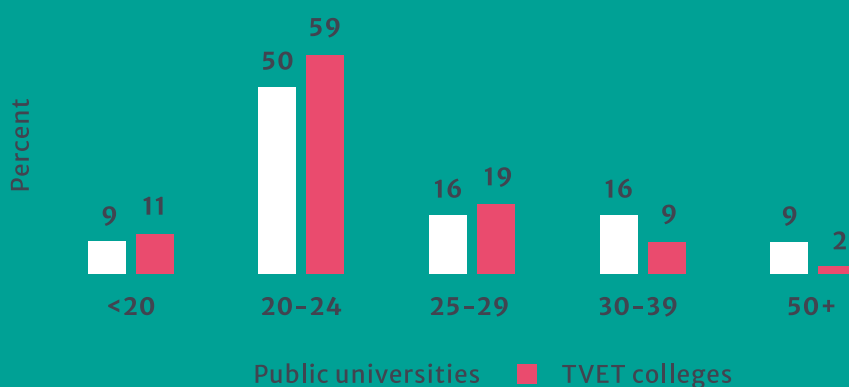
♀ **58%** ♂ **42%**

Many part-time students
FTE enrolment **305 659**
Students with disabilities **0.3%**

Provincial share of students

Eastern Cape **10%**
Free State **8%**
Gauteng **26%**
KwaZulu-Natal **17%**
Limpopo **12%**
Mpumalanga **7%**
North West **5%**
Northern Cape **2%**
Western Cape **13%**

AGE DISTRIBUTION OF STUDENTS



*Note: graduation rates can be regarded as a proxy for throughput rates. All figures derived from Statistics on post-school education and training in South Africa 2018. Dept of Higher Education and Training, 2020

1.8

CONTINUITY IN A TIME OF CHANGE

During this period of major change, HIGHER HEALTH and its partners endeavoured to sustain the established activities and services that have been developed over the years on campuses across the country: screening and testing for health conditions, health promotion activities, condom distribution, clinic services (including HIV treatment and SRH services), promotion of the rights of vulnerable groups, and strengthening the campus response to GBV.

The volume of activities was generally lower than in the previous year. This was partly because Second Curriculum activities absorbed considerable time and partly because the far-reaching organisational changes at HIGHER HEALTH demanded a great deal of attention. In addition, some campuses were once again disrupted by student and staff protests – particularly during the opening weeks of the academic year, which are normally peak health promotion and screening periods.

FIRST THINGS FIRST HEALTH AND WELLNESS DAYS

From January to December 2019, there were a total of 738 First Things First activations at universities and TVET colleges. Many activations ran over a period of several days, and the total number of activations translated to 1 779 individual health and wellness days. TVET colleges accounted for 55% of these days and universities for 45%. On these occasions, students have the opportunity to test for HIV, measure their blood sugar and blood pressure, and undergo screening for TB and STIs. A variety of health promotion activities are organised by peer educators. Partners in the public health services and civil society are critical to the success of these events.

The total number of individuals reached through face-to-face interaction with peer educators in 2019 was 704 988. Many of these engagements occur during health days, but also take the form of dialogues and individual discussions.

HIV TESTING AND TB AND STI SCREENING

Extensive student testing for HIV and screening for TB and STIs takes place during First Things First activities as well as through campus health clinics. Many students find it easier to test during activations and health days, and the decline in these activities is reflected in a downturn in screening and testing figures.

In 2019 a total of 155 719 students tested for HIV, 169 815 were screened for TB and 166 660 for STIs. These figures represent a decrease of about 30% on the numbers reached in 2018. While the HIV test produces a definitive result, screening for TB and STIs identifies symptoms that may indicate infection and students are referred for further investigation.

The percentage of students testing positive for HIV (1.1%) is very low compared to national HIV prevalence in the equivalent age group. It may be that those most vulnerable to infection do not opt for testing and HIGHER HEALTH is addressing this in two ways:

- Incorporating self-assessment of risk into the Second Curriculum.
- Introducing HIV self-testing – or HIV self-screening, as it is referred to in this country – as it has been shown that the privacy of this method appeals to reluctant testers. About 500 university students opted for HIV self-screening in 2019.

HIGHER HEALTH will strive to regain lost ground in terms of the proportion of students who are aware of their HIV status. Proactive testing contributes to achieving South Africa's 95-95-95 targets, in terms of which we aim to ensure that 95% of people living with HIV (PLHIV) know their status, that 95% of this group receive antiretroviral therapy (ART), and 95% of those on treatment achieve and sustain viral suppression. HIV testing is also a sound foundation for promotion of various forms of biomedical and behavioural methods of HIV prevention.

Table 1: Screening and testing during 2019

HIV testing		STI screening	
Number tested	155 719	Number screened	166 660
Number testing positive	1 739	Referred for further investigation	3 680
Number referred for ART	1 917		
TB screening		*NCD screening	
Number screened	169 815	Number screened	133 147
Number referred for lab testing	961	Referred for further investigation	183

*This refers mainly to diabetes and hypertension screening

CONDOM DISTRIBUTION

The value of a consistent supply of condoms to students cannot be overemphasised as condoms remain the only technology that prevents HIV, most other STIs and pregnancy.

About 41% of all male condoms and 35% of all female condoms were distributed at TVET colleges, but only 18% of lubricant reached TVET colleges.

Recent progress in offering woman students convenient access to contraception was largely sustained in 2019 and the number who seek pregnancy tests at campus facilities has increased.

Table 2: SRH products and services provided in 2019

Condom distribution		Contraception	
Male condoms	7 292 816	Referred for contraceptives	3 294
Female condoms	387 360	Long-term contraceptives provided	48 283
Lubricant	316 465	Emergency contraception provided	108
Reproductive health		Medical male circumcision (MMC)	
Pregnancy tests	3 822	Referred for MMC	858
Pregnancies confirmed	1 274	MMC performed	145
Referred for termination	324		

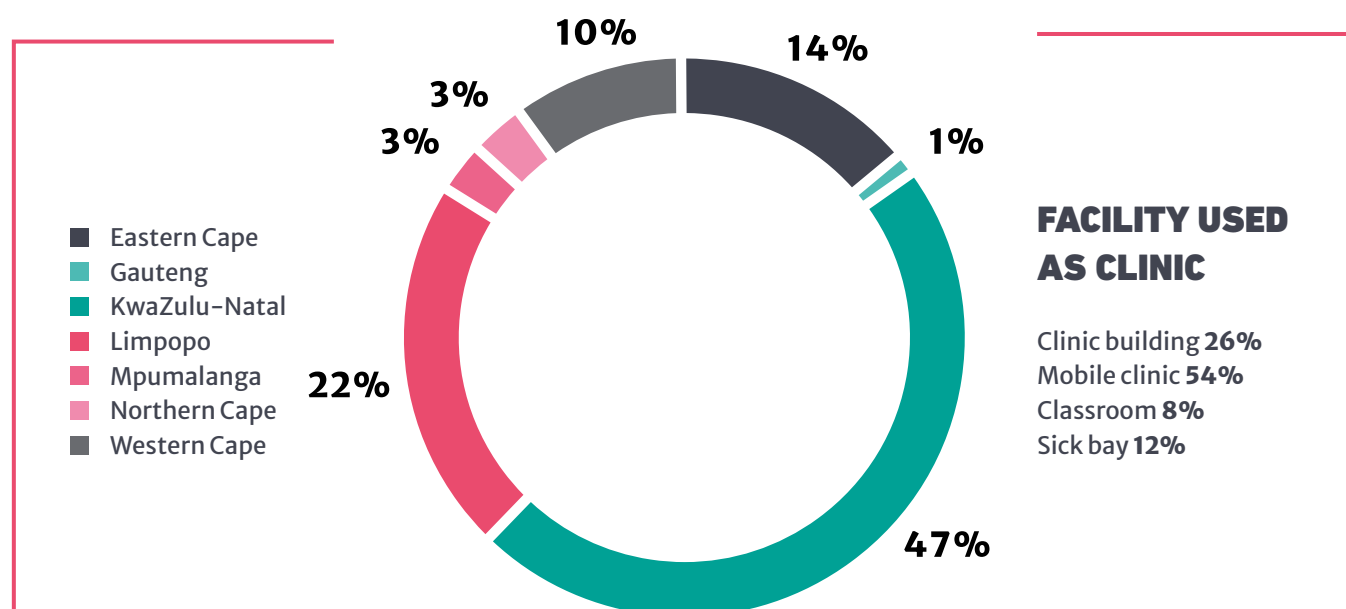


ON-CAMPUS CLINIC FACILITIES

All public universities have campus clinics, though these are not always available on every campus. At TVET colleges, the public health sector has assisted in

setting up campus facilities. HIGHER HEALTH's focus during 2019 was strengthening clinics, rather than expanding their number. Our concern was to ensure consistent supplies of equipment, medication and consumables from departments of health and NGOs.

Figure 5: Type of clinic facility at TVET colleges and provincial distribution 2019



MENTAL HEALTH

During 2019/20, HIGHER HEALTH's mental health activities were geared to building the capacity of CHWCs, peer mentors and peer educators to raise awareness of mental health issues experienced by young people and challenge the stigma that continues to surround mental illness. In effect, we have begun to break a destructive silence – but now we have to gear up to provide answers.

With the introduction of self-screening for mental health risk, there is a pressing need to develop mental health services to which students at highest risk can be referred for help. We are aware this will not be easy. Some institutions – mostly universities – offer psychological services but most do not.

Mental health is a specialised area of healthcare that is under-resourced in the public health sector. While provincial and district health services have proved willing partners in the areas of HIV and TB, non-communicable diseases and SRH, they have less capacity to share when it comes to mental health – and necessity has produced another “first” for HIGHER HEALTH.

We have been working with the Department of Social Development to place some 80 intern social workers at TVET colleges to provide a basic level of psychosocial support. At the same time, we are exploring whether psychosocial services funded by USAID can extend their reach to include TVET college students.

FUTURE BEATS RADIO AND SOCIAL MEDIA PROGRAMME

Campus and community radio stations continue to play a valuable role in enhancing students' understanding of health-related matters and providing useful, actionable information. During 2019 a total of 13 stations participated in the Future Beats programme, receiving a small grant and training for their journalists, producers and managers. The training has three main elements:

- Factual education on the health focus areas that guide all HIGHER HEALTH activity.
- Building of radio production skills to present this material in a compelling manner.
- Technical and design training to enable stations to use social media to complement radio production.

The stations follow monthly themes for programme content which is selected by HIGHER HEALTH. The funded stations have a combined audience of nearly 800 000.

A sub-group of stations produced additional programmes aimed at AGYW under a partnership with Johnson & Johnson, which manages the Dreams Thina Abantu Abasha initiative in the provinces of Gauteng and KwaZulu-Natal.

In January 2020, contracting processes for partnerships with a further seven stations commenced.

1.9

LEADERSHIP, ADVOCACY AND NATIONAL COORDINATION

The development of a holistic health and wellness programme within the higher education sector in South Africa is pioneering work and advocacy for this programme is central to its success. Fortunately, we have the support of respected individuals within both the health and education fields who are willing to share this responsibility with us.

The Deputy Minister of Higher Education and Training, Mr Buti Manamela, has been one of our most consistent champions and his support has undoubtedly helped to build trust in the HIGHER HEALTH enterprise.

Public events at universities and TVET colleges are valuable opportunities for us to present our unfolding model of practice to important stakeholders – including leaders within these institutions, and representatives of provincial and district government, civil society organisations and funding bodies – and strengthen our relationships with them.

Opportunities to participate in international conferences and symposia have been invaluable in extending HIGHER HEALTH's access to expertise and other resources. Important events that took place during the reporting period are described briefly below.

Table 3: Major events hosted and/or addressed by HIGHER HEALTH in 2019/20

Event	Host organisation	Main speaker(s)
Events organised by HIGHER HEALTH		
First Things First wellness day and dialogue on GBV April 2019	University of Mpumalanga (UMP)	Deputy Minister Manamela
Satellite session at SA AIDS Conference in Durban entitled Youth, health and wellness: innovation towards controlling the HIV epidemic June 2019	HIGHER HEALTH	Panel members: SA AIDS Conference Chair Prof Refilwe Phaswana-Mafuya, SA Union of Students (SAUS) President Mr Misheck Mugabe, South African National AIDS Council (SANAC) Co-Chair Ms Steve Letsike, UNFPA Regional Deputy Director Ms Beatrice Mutali, and UNAIDS senior advisor Dr Ali Feizzadeh
Dialogue on GBV at Conference of International Association of Women Judges August 2019	UMP and SA chapter of International Association of Women Judges	HIGHER HEALTH CEO Dr Ramneek Ahluwalia, UMP Vice-Chancellor Prof Thoko Mayekiso, Chair of Council of Nelson Mandela University Dr Nozipho January-Bardill, founder of the Khuhluka Movement Ms Josina Machel, and SAUS President Mr Misheck Mugabe
First Things First wellness day and dialogue on GBV October 2019	University of KwaZulu-Natal	Deputy Minister Manamela

Table 3: Major events hosted and/or addressed by HIGHER HEALTH in 2019/20 (Cont.)

Event	Host organisation	Main speaker(s)
Events organised by HIGHER HEALTH		
First Things First activation on STI prevention and treatment January 2020	College of Cape Town	Western Cape Minister of Health Dr Nomafrrench Mbombo and Minister for Social Development Sharna Fernandez
First Things First wellness day and dialogue on GBV February 2020	Goldfields TVET College, Welkom	Deputy Minister Manamela and Deputy Minister for Women, Youth and Persons with Disabilities Prof Hlengiwe Mkhize
Major external events addressed by HIGHER HEALTH leadership		
UN General Assembly session August 2019	United Nations, New York	HIGHER HEALTH CEO presented practice model
10th Pan-African TVET College Conference October 2019	Pan-African TVET College Conference	HIGHER HEALTH CEO addressed conference and participated in panel discussions
International Conference on Population and Development November 2019	UNFPA and the governments of Denmark and Kenya	HIGHER HEALTH CEO was part of the South African delegation and addressed the conference
Conference on Making Tertiary Education #safetolearn November 2019	UK Department of International Development and Foreign and Commonwealth Office	HIGHER HEALTH CEO participated in panel discussion on challenges of implementing programmes
Namibian conference on the population dividend November 2019	UNAIDS, UNFPA and UNESCO	HIGHER HEALTH CEO was invited as a speaker and panel member on sexual and reproductive health rights of young people
5th United Nations Conference on Safer Cities and Public Spaces for Young Women and Girls February 2020	United Nations	HIGHER HEALTH CEO Dr Ramneek Ahluwalia addressed a conference session

1.10

SUSTAINING AND EXPANDING PARTNERSHIPS

Partnerships are the bedrock of HIGHER HEALTH's ability to develop health programmes for students across South Africa. Without partners who are passionate about improving youth health, our ideas would never come to fruition. Therefore, our managers at head office invest

much of their time in expanding and sustaining partnerships. The establishment of HIGHER HEALTH's regional and provincial offices will further strengthen our interaction with partners.

UNIVERSITIES AND TVET COLLEGES AS PARTNERS

Our primary partnerships are with the universities and TVET colleges that constitute the higher education and training sector. These are mission-critical relationships and we value them immensely. At each institution we interface with an institutional coordinating committee and this practice will continue into the future. The fact that we now have CHCWs at every TVET college does not in any way substitute for committed, active coordinating committees.

HIGHER HEALTH also facilitates occasional regional workshops and national forums for various staff members of universities and colleges. These enable participants to exchange information, contribute to the development of HIGHER HEALTH strategies and programmes, and address common challenges. Other partners in government, civil society and the funding community often participate in the workshops. More detail on this area of work is included in Section 2.

Over the years, HIGHER HEALTH has endeavoured to involve students not only in the execution of health and wellness programmes but also in their planning and development. This has been challenging because of the regular turnover of leadership in student structures. However, progress has been made: 2019 saw the establishment of a dedicated forum for representatives of SAUS and the South African Further Education and Training Student Association (SAFETSA). However, since SAFETSA's elections were declared null and void, HIGHER HEALTH interacted with individual student representative councils at TVET colleges. The forum met in July and October and SRC presidents from all 26 public universities participated.

PARTNERSHIP WITH DEPARTMENT OF HIGHER EDUCATION AND TRAINING

HIGHER HEALTH's roots are in the Department of Higher Education and Training and, although we are now structurally independent, this relationship remains very significant. The interest and support of the department extend well beyond formal representation on our board. Deputy Minister Manamela continues to be a visible presence at important HIGHER HEALTH events. In turn, HIGHER HEALTH strives to understand evolving policy within the department so that our activities are well aligned with the strategic directions taken.

PARTNERSHIPS WITH FUNDING BODIES

HIGHER HEALTH appreciates that universities and colleges need to direct their available funding mainly to their core functions of learning and teaching, and research. We believe our ability to mobilise additional funding for the health and wellness needs of students is a critical aspect of our mission.

We have been extremely fortunate in finding common cause with a number of important funding institutions. The key to this has been our shared concern for the development and wellbeing of young people. Many funders are keen to invest in youth development – for a range of reasons – and HIGHER HEALTH has structured processes for reaching an important section of the youth population.

During 2019/20 we were fortunate to retain some of our long-term funders and form new partnerships with others.

PARTNERSHIPS WITH HEALTH SERVICES

HIGHER HEALTH's dream of bringing relevant, user-friendly healthcare services to students right on their doorstep is gradually being realised through partnerships with various provincial and district health services. These public health services have assisted in a number of ways:

- Boosting existing campus health clinics by providing medical supplies, including test kits, medicines and contraceptives.
- Running public health clinics on campus either in mobile units, in available space at institutions or in purpose-built clinics provided by provincial health departments.
- Training health professionals employed directly by university and college health clinics so that they can expand their services to meet the needs of students. For example, training has been provided in administering ART and primary mental healthcare.

The value of this assistance cannot be overstated. We would like to think that public health services benefit in return by gaining increased access to young people, a section of the population who are known to be reluctant health service users.

In addition to public health sector partners, HIGHER HEALTH enjoys collaborative relationships with literally hundreds of NGOs and CBOs working in the health and wellness field.

1.11

GOVERNANCE

The HIGHER HEALTH Board of Directors, chaired by Prof Wim de Villiers, Vice-Chancellor of Stellenbosch University, met three times during the 2019/20 financial year. The Board's focus during the year was developing a medium-term strategy for the organisation and overseeing the rapid expansion of the organisation.

A small number of key stakeholders are represented on the Board (USAf, the South African College Principals

Organisation (SACPO) and the DHET) while several positions are held by individuals who bring specific expertise to the governance team. A full list of members appears in the annual financial statements towards the end of this report.

The Board has three specialised committees which met as necessary during the year.

Table 4: Committees of HIGHER HEALTH Board

Committee	Chairperson	Number of meetings
Audit & Risk and Finance & Investment Committee	Mr Theo Madurai	Three
Human Resources and Remuneration Committee	Dr Ahmed Bawa, USAf representative on the Board	Three
Technical Advisory Committee	Prof Nancy Refiloe Phaswana-Mafuya, Deputy Vice-Chancellor Research and Innovation, North West University	No meetings. Committee is being reconstituted

1.12

MANAGEMENT AND HUMAN RESOURCES

As indicated earlier in the report, the total staff complement of HIGHER HEALTH expanded dramatically during 2019/20, from a total of 31 members to more than 90. This growth created a highly dispersed body of employees, with 62 CHCWs situated on TVET college and university campuses. The establishment of regional and provincial offices was, to a large extent, prompted by the need to manage CHCWs effectively and support them so that they perform to their best ability.

HIGHER HEALTH used a specialist recruitment service to identify potential candidates across the country. Shortlisting of candidates, interviewing and final selection were driven internally. We will be prioritising the development of in-house HR management capacity in the coming year.

Expansion has gone hand-in-hand with application of B-BBEE principles and HIGHER HEALTH has created jobs for young people and women, in particular.

Figure 6: Employees by occupational category

- Executive & senior management
- Middle management
- Professional & technical
- CHCWs
- Administrative

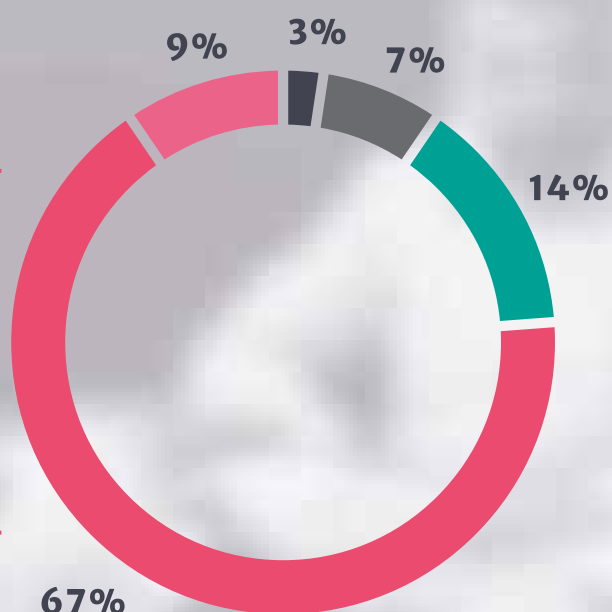
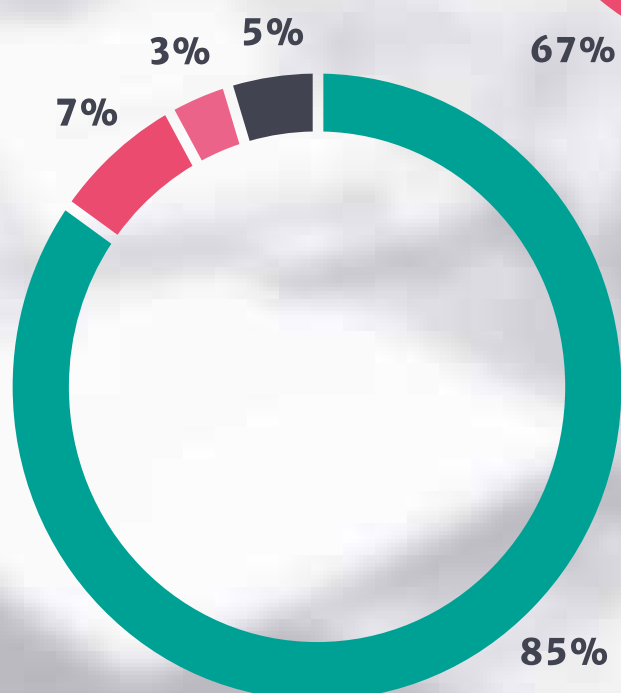


Figure 7: Employees by racial category

- African
- Coloured
- Indian
- White



1.13

ACKNOWLEDGEMENTS

The list of organisations and individuals who have contributed to HIGHER HEALTH's initiatives grows longer each year as our activities expand. To each and every one making a contribution, we extend our sincere thanks.

While I cannot possibly mention every contribution, I would like to acknowledge major supporters.

We are indebted to the Minister of Higher Education, Science and Technology, Dr Blade Nzimande, and Deputy Minister Manamela for their steadfast support of our work. We also thank officials in the DHET for facilitating the HIGHER HEALTH's activities in countless ways.

We count ourselves fortunate to be in partnership with the country's public universities and TVET colleges. Their participation takes many forms and emanates from diverse quarters of campus communities, including management of institutions, staff members of all descriptions and many students. We appreciate above all the energy, creativity and compassion that many of you bring to the programme. The public health sector, from the national Department of Health to provincial health departments and district health offices, has played a critical role in resourcing campus health services and supporting health and wellness days. It is very difficult to quantify their total contribution but we are certain that it is irreplaceable. International and South African funding agencies have invested generously in our work and enabled

us to grow. HIGHER HEALTH appreciates the relationships that have deepened during the course of our joint ventures and looks forward to continued collaboration.

HIGHER HEALTH has always benefited from the experience and insight that resides in the vast number of NGOs and CBOs active in the health and wellness space. We are proud to be part of this non-profit sector and have benefited from collegial sharing of knowledge and experience.

Finally, there is the HIGHER HEALTH "family" – or perhaps it is now more of a clan – comprising members of our Board of Directors, its sub-committees and employees of the organisation.

I am deeply grateful to the Board, and especially to our Chairperson, Prof de Villiers, for the time they have devoted to our organisation. Not one member has time to spare – and yet you make time to give HIGHER HEALTH your best advice and guidance. This is sincerely appreciated.

Finally, I extend my thanks to our employees, new and old, who have shared the responsibility of steering HIGHER HEALTH into a new age. It has been a challenging time for everyone and has taken many of us out of our personal comfort zones. Each individual has played a part but I would be remiss if I did not acknowledge the efforts of a small group of head office managers who have made a super-human effort to keep the process of change on track. Your dedication is recognised and truly valued.

DR RAMNEEK AHLUWALIA
CHIEF EXECUTIVE OFFICER



02

SECTION 2

PROGRAMME PERFORMANCE

This section describes the performance of the University and TVET College Programmes, which follow the same model but are supported by HIGHER HEALTH in slightly different ways. While we assist TVET colleges directly through the placement of CHCWs on their campuses and payment of stipends to peer mentors, we support universities through grants that can be used to implement various aspects of the programme.

This section also provides details of the Future Beats radio-based youth development programme and other communication and stakeholder activities.

A note on reporting periods:
Although this report covers the financial year 1 April 2019 to 31 March 2020, programme activities are reported for the 2019 academic year, with some supplementary information provided for the first quarter of 2020.

2.1

UNIVERSITY PROGRAMME

The HIGHER HEALTH programme is implemented at all 26 public universities and, in most instances, it is a mature programme that was initiated two decades ago. It is characterised by peer-led health and wellness activities and by the daily availability of campus health clinics at most institutions. Peer mentors, campus health coordinators and healthcare professionals are usually employed as part of the staff establishment of universities.

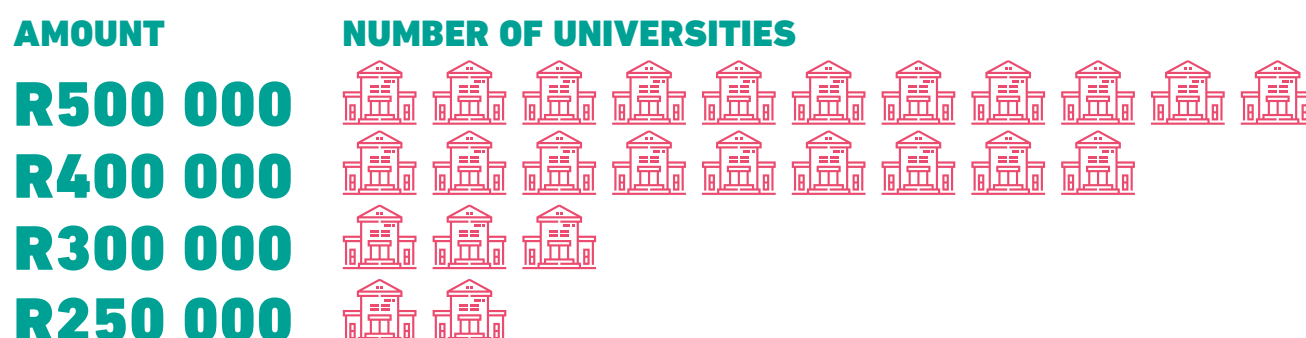
At new universities – Mpumalanga University and Sol Plaatje University – as well as some that remain underdeveloped due to historic disadvantage – Walter Sisulu University and Mangosuthu University of Technology – HIGHER HEALTH provides support over and above the general grant.

2.1.1 GRANTS TO UNIVERSITIES

A total amount of R10.5m was allocated in grants to universities for purposes of implementing the HIGHER HEALTH programme over the period 2019 to 2021. All universities are eligible and grants were approved for 25 of them in 2019, with the first payment of 30% effected immediately. Universities have some discretion as to how they use the grant, but have targets to meet in terms of:

- Testing for HIV and screening for TB and STIs.
- Student participation in Second Curriculum sessions.
- Risk assessments conducted to prevent the development of mental and sexual health problems.

Figure 8: Grants to universities 2019–2021



2.1.2 PEER-TO-PEER MOBILISATION AND EDUCATION

HIGHER HEALTH's programme depends on student power to drive health and wellness activities on campuses. Over 4 000 peer educators, who are recruited and trained each year, are at the heart of this.

During 2019, peer educators at universities interacted with a total of 557 902 students during health and wellness days, campus dialogues on health-related issues, and in more private discussions.

2.1.3 FIRST THINGS FIRST ACTIVITIES

First Things First activations are highly visible health events that bring health screening and testing out of clinical settings into the heart of campus life and help build health literacy among students. Each activation may comprise several individual health days, often held at different locations within the same university.

The University Programme encourages institutions to ensure that every campus has more than one health day a year and in 2019 the total number of activations was 159, which translated into 795 individual wellness days.

Performance against targets for activations and health days was disappointing in 2019, with only a handful of universities meeting or exceeding targets. This was partly due to a delay in the availability of grant funding from donors and partly due to cancellation of some First Things First activities due to student and staff protests.

Table 5: University screening and testing during 2019

HIV testing		STI screening	
Number tested	72 977	Number screened	83 937
Number testing positive	680	Referred for further investigation	2 584
Number referred for ART	846		
TB screening		*NCD screening	
Number screened	86 037	Number screened	60 244
Referred for further testing	671	Referred for further investigation	90

**This refers mainly to diabetes and hypertension screening*

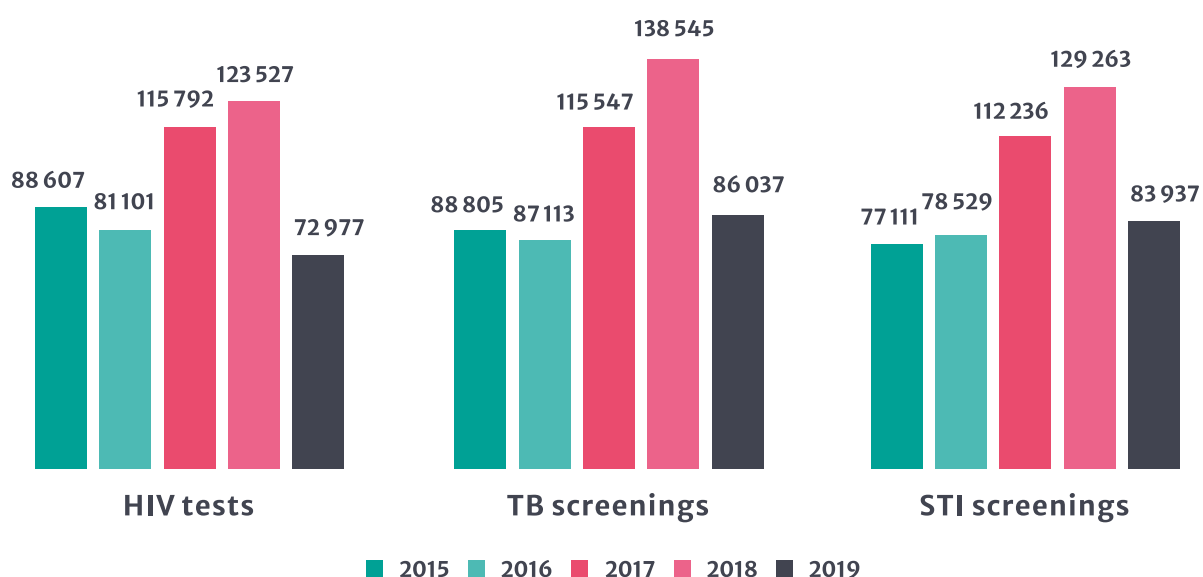
The numbers of tests and screenings performed at universities during 2019 were substantially lower than in 2018. For example, there was a 41% decline in HIV testing mainly as a consequence of fewer activations and health days.

The profile of students opting to test was consistent with previous years. About 12% were first-time testers and six out of 10 testers were women. While the university

sector has more women than men enrolled as students, the skewed gender ratio for testing and screening also reflects a more widely observed reluctance among men to use health services.

Only 0.93% of students testing on campus were HIV-positive, an extremely low rate compared to national prevalence for this age group. About 3% of students screened for STIs warranted further investigation.

Figure 9: Five-year trend in HIV testing, TB screening and STI screening at universities



2.1.4 SEXUAL AND REPRODUCTIVE HEALTH

HIGHER HEALTH encourages a youth-friendly rights-based approach to SRH and attempts to create a climate in which sexual health can be openly discussed. Campus health clinics offer STI treatment, contraceptive services, pregnancy testing and screening for various cancers. They refer students for antenatal care, termination of pregnancy, medical male circumcision and specialised treatment.

There is a global trend towards the integration of SRH and HIV services, and campus health clinics are a natural space for such integration.

We aim to ensure that no young woman's tertiary education is cut short by an unplanned pregnancy. While effective contraception is the preferred approach, safe termination of pregnancy or a well-managed pregnancy and subsequent return to studies are options campus clinics facilitate.

Peer educators play a vital role in creating awareness of and demand for these services. The distribution of condoms and access to other SRH and HIV-prevention products are critical to the prevention of infections and unintended pregnancy. In addition to condoms, university clinics are increasingly offering PrEP as a method of HIV prevention that works equally well for students of all sexes, genders and sexual orientations. Nearly 600 students were offered PrEP, and about 44% opted to try this method. This was a substantially higher proportion than at TVET colleges.

Comprehensive services for survivors of rape include emergency contraception, post-exposure prophylaxis of HIV (PEP), prevention of other STIs and counselling services.

Unfortunately, distribution of condoms on university campuses was less effective in 2019 than the previous year. A total of 4.3 million male condoms were distributed, 39% fewer than the 7 million distributed in 2018.

Table 6: SRH products and services provided in 2019

Condom distribution		Contraception	
Male condoms	4 333 683	Referred for contraceptives	2 244
Female condoms	212 113	Long-term contraceptives provided	259 358
Lubricant	259 358	Emergency contraception provided	108
Reproductive health		Medical male circumcision (MMC)	
Pregnancy tests	1 214	Referred for MMC	42
Pregnancies confirmed	1 252	MMC performed	141
Referred for termination	324		

2.1.5 PSYCHOSOCIAL SERVICES

The development of accessible and relevant psychosocial support for university students is still in its infancy, as the figures show. It will be driven, to a large extent, by

students becoming more aware of mental health and increasingly able to identify their own needs. As delivery of the Second Curriculum gathers momentum at universities, the demand for psychosocial support will increase.

Table 7: Psychosocial services at universities 2019

	Screened for service need	Referred to external service	Assisted on campus
Psychosocial support for GBV	41	0	8
Mental health counselling	785	35	37
Counselling for alcohol & substance use	7	57	231

2.1.6 FORMALISATION OF THE SECOND CURRICULUM

HIGHER HEALTH prioritised TVET colleges for the building of the new delivery platform for the Second Curriculum (see page 28 for description). As a result, only 23 university staff members received training on the facilitation of

Second Curriculum modules in 2019 and early 2020. The intention was to intensify training for university staff members from April 2020.

Despite limited capacity building, several thousand students benefitted from Second Curriculum sessions in the first three months of 2020 and completed self-administered risk assessments, as indicated in the table.

Table 7: Psychosocial services at universities 2019

	HIV, TB, STIs and SRH	GBV and domestic violence	Mental health
Attendance at sessions	10 126	9 688	9 688
Completion of risk screenings	16 659*	736	2 321

*Some of these risk assessments were performed during individual interactions between peer educators and students.

2.1.7 UNIVERSITY LIAISON

Sound, interactive communication with higher education institutions is critical to the planning and implementation of the HIGHER HEALTH programme and it occurs at three levels in respect of universities: individual meetings, regional forums and national forums.

In 2019, members of the University Programme staff initiated and conducted 52 meetings with individual universities – that is, two per university. These meetings are designed to facilitate programme management and ensure financial accountability.

Various provinces are clustered into regions and the target is two meetings a year for each region. Universities in all provinces, except the Free State, attended at least one regional meeting. These gatherings are more strategic than operational and are typically attended by deans of students, SRC presidents, grant managers, relevant heads of department, campus security, provincial departments of health and social development, and other government departments, as relevant.

HIGHER HEALTH has established specialised national forums for various managers and leaders at universities across the country. Not all of these forums convened in 2019/20, but the following were active:

- The Forum for Deans of Students met three times.
- The Forum for Campus Security Managers met once.
- The Forum for Student Leadership (SAUS) met twice and SRCs from all universities were represented.
- National Forum of Executive Committees, comprising managers of campus health services, managers and practitioners in the area of SRH and employees responsible for gender transformation, met in May 2019 to consider capacity development for first responders to mental health problems and GBV, campus sensitisation on GBV, safe room provision at clinics, and protocols for response to rape.



2.2

TVET COLLEGE PROGRAMME

There are 50 TVET colleges in South Africa with a total of 259 campuses distributed across all provinces. Coordinating and guiding health and wellness

activities across this large number of sites is a challenge and HIGHER HEALTH's new regional configuration has been designed largely to meet this challenge.

Table 9: Number of TVET colleges and campuses by HIGHER HEALTH region

Region	No of colleges	No of campuses	No of CHWCs
Northern Region Gauteng, Limpopo and North West	18	80	19
Eastern Region Free State, KwaZulu-Natal and Mpumalanga	16	96	22
Cape Region Eastern Cape, Northern Cape and Western Cape	16	83	19

Note: Two more CHWCs have been placed at Walter Sisulu University and Mangosuthu University of Technology.

2.2.1 HUMAN RESOURCES SUPPORT

The sheer number of TVET campuses and their dispersed location makes coordination of the HIGHER HEALTH programme in this sub-sector demanding. Furthermore, college campuses generally have a smaller enrolment than university campuses and often lack the student support services that are part and parcel of universities. These factors contributed to the decision by HIGHER HEALTH to support TVET colleges more directly, through the placement of CHWCs on at least one campus of each college.

CHWCs do much more than coordinate the activities of others; they are equipped with the skills to provide key health and wellness services. Among other things they are trained to:

- Facilitate sessions on all modules of the Second Curriculum.
- Perform HIV testing.
- Conduct risk assessments.
- Provide lay counselling.

In addition, rather than make grants available to TVET colleges, HIGHER HEALTH has begun to pay a stipend to staff members who take on the role of peer education mentors as this involves responsibilities beyond their regular duties. Applicants for peer mentor positions are generally life orientation lecturers, student liaison officers or career guidance officers all of whom have a vested interest in the health and wellbeing of students. As of 2020, we have 255 peer mentors at TVET colleges.

2.2.2 PEER-TO-PEER MOBILISATION AND EDUCATION

During 2019, inter-personal communication activities by peer educators at TVET colleges reached a total of 147 086 students.

Preparatory work for the programme of Second Curriculum sessions was a major limiting factor on the delivery of other peer education activities in 2019.

Table 10: TVET college screening and testing during 2019

HIV testing		STI screening	
Number tested	82 742	Number screened	82 723
Number testing positive	1 059	Referred for further investigation	1 096
Number referred for ART	1 071*		
TB screening		NCD screening **	
Number screened	83 778	Number screened	72 903
Referred for further testing	290	Referred for further investigation	93

*This includes all known HIV-positive students referred for ART **This refers mainly to diabetes and hypertension screening

In 2019, for the first time since TVET colleges joined our programme, they conducted more HIV tests in a year than universities did. The TVET College Programme also managed to sustain a high level of activations and health days, recording a relatively low dip in testing and screening numbers (15% for HIV and 13% for TB) compared to 2018.

2.2.3 FIRST THINGS FIRST ACTIVATIONS

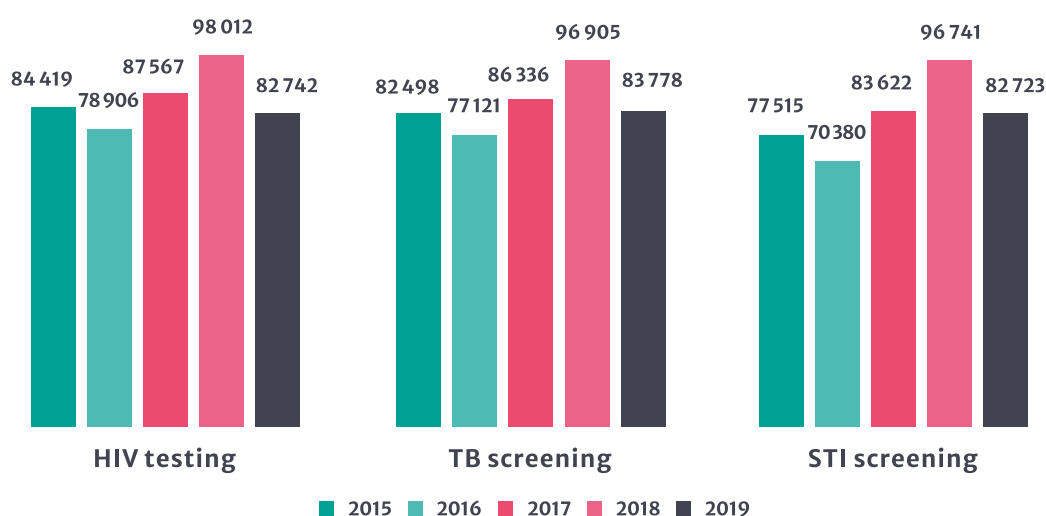
A total of 579 First Things First activations took place on TVET college campuses in 2019, translating into 984 individual health and wellness days. This means that the average number of health and wellness days per campus came close to four per year.

While HIV testing and TB and STI screening are done both during routine clinic visits and activations, the option of testing and/or screening during a wellness day is most popular. The total number of tests and screenings performed are reflected below.

First-time HIV testers accounted for 14% of the total and the gender ratio among students undergoing testing and screening was strongly skewed toward women. The percentage of students who tested HIV-positive was 1.27%, which is well below the national HIV prevalence rate for the age group.

TVET colleges slightly expanded their screening for non-communicable diseases in 2019.

Figure 10: Five-year trend in HIV testing and TB and STI screening at TVET colleges



2.2.4 SEXUAL AND REPRODUCTIVE HEALTH

The approach to SRH at TVET colleges is the same as that at universities, a combination of rights-based awareness-raising and education, and provision of youth-friendly services on campus. In the TVET college environment, HIGHER HEALTH has played an instrumental role in the development of campus clinics at which SRH services are offered. We have helped forge relationships with provincial and district health authorities that have ultimately provided regular clinic services on a large number of campuses.

Peer educators are invaluable in promoting SRH, especially at colleges in rural areas where traditional norms prevail and inter-generational discussions of sex and sexuality are uncommon.

The extraordinary value of condoms as the multitool that prevents HIV, most other STIs and pregnancy was noted earlier in this report. After an exceptional year in 2018, when 4.1 million male condoms were distributed, recorded condom distribution at TVET colleges slipped to under 3 million in 2019. There was very limited availability of female condoms and lubricant.

Table 11: Uptake of SRH services at TVET colleges 2019

Condom distribution		Contraception	
Male condoms	2 959 133	Referred for contraceptives	1 050
Female condoms	175 247	Long-term contraceptives provided	12 275
Lubricant	57 107	Emergency contraception provided	0
Reproductive health		Medical male circumcision (MMC)	
Pregnancy tests	2 608	Referred for MMC	816
Pregnancies confirmed	22	MMC performed	4
Referred for termination	0		

2.2.5 FORMALISATION OF THE SECOND CURRICULUM

All CHWCs and 255 peer mentors received training on the modules of the Second Curriculum. This equipped them to take responsibility for facilitating Second Curriculum sessions from January 2020 and introducing the self-assessment of health risks to students.



Table 12: Participation in Second Curriculum activities at TVET colleges January – March 2020

	HIV, TB, STIs and SRH	GBV and domestic violence	Mental health
Attendance at sessions	20 917	23 986	23 986
Completion of risk assessment	47 851	20 681	20 681
Assessed at High Risk	12%	15%	13%

Note: a large number of risk assessments were also performed during individual interactions between peer educators and other students.

2.2.6 PSYCHOSOCIAL SERVICES

Where self-screening indicated students were at high risk of GBV or experiencing mental health problems, they were linked to psychosocial care provided by partner organisations that visit campuses or campus counselling units, where these exist.

2.2.7 PROGRAMME FOR ADOLESCENT GIRLS AND YOUNG WOMEN

A Global Fund grant, that commenced in October 2019, will enable HIGHER HEALTH to concentrate interventions in a sustained manner on a substantial group of AGYW who volunteer to be part of this programme.

The programme, which is described quite extensively in the CEO's Overview, will be led by 30 CHWCs based at the selected TVET colleges. By tracking the participants

across their college careers, HIGHER HEALTH will obtain a better understanding of the impact of our interventions.

Prevention interventions included offering an HIV test and condoms, providing information on HIV, SRH and GBV, conducting risk screening for HIV and GBV, and individual referrals for services. HIGHER HEALTH envisages that offering PrEP and supporting students to take the daily dose will become an important aspect of the AGYW programme.

The AGYW programme sites will receive vending machines which will enable woman students to gain free access to HIV prevention products, pregnancy tests and sanitary protection.

CHWCs responsible for this programme have been trained to administer the HIV finger-prick test, to secure linkage to care for all HIV-positive students, and to enable students to administer the risk assessment.

Table 12: Participation in Second Curriculum activities at TVET colleges January – March 2020

Province	District	TVET college	No of CHWCs
Eastern Cape	Nelson Mandela Bay Metro	Port Elizabeth	3
		Eastcape Midlands	2
	OR Tambo	KSD	2
Free State	Thabo Mofutsanyane	Maluti	3
Gauteng	Tshwane	Tshwane North	2
		Tshwane South	1
KwaZulu-Natal	King Cetshwayo	Umfolosi	2
	Zululand	Mthashana	2
Limpo	Sekhukhune	Sekhukhune	2
Mpumalanga	Gert Sibande	Gert Sibande	3
	Ehlanzeni	Ehlanzeni	2
North West	Bojanala	Orbit	2
Western Cape	City of Cape Town	College of Cape Town	4

This programme is strongly target driven and peer mentors will take charge of demand generation through their teams of peer educators. Performance has been uneven thus far: while HIV testing numbers were not up-to-speed by March 2020, coverage of various HIV prevention activities was ahead of the pro-rata target for five months.

Perhaps of greatest concern, in terms of programme performance, is that models suggest that 4.5% of the total number tested would be both HIV-positive and linked to care, while the actual achievement is far lower at 0.3%. The challenge appears to be reaching AGYW who are at highest risk, encouraging them to undergo testing and enabling them access to all other benefits of the programme.

Table 14: Key indicators for the Global Fund AGYW Programme at TVET colleges

Coverage indicator	Year 1 target (12 months)	TVET college (5 months)
Number tested for HIV and received results	14 378	4 094
Number linked to HIV care	647 (4.5% of total tested)	14 (0.3% of total tested)
Number reached with HIV prevention activities	5 991	3 240

2.2.8 LIAISON WITH TVET COLLEGES

HIGHER HEALTH managers and CHWCs visit TVET colleges regularly in order to coordinate health and wellness activities. In 2019, a total of 648 such visits took place and they served a number of purposes:

- To coordinate and review First Things First health and wellness days.
- To meet with partners in the provincial and district health services.
- To provide mentorship to CHWCs.

Six provincial forums of TVET colleges were convened during the year to provide strategic direction for programme implementation. They were attended by college principals and their deputies, campus managers, representatives of student support services and student representative councils, and officials of the DHET, DOH, DSD and SAPS as well as other government departments and partner organisations.

2.3

FUTURE BEATS YOUTH DEVELOPMENT MEDIA PROGRAMME

The Future Beats Programme, which leverages partnerships with campus and community radio stations in order to stimulate discussion among students about health and related social matters, has been a solid pillar of HIGHER HEALTH over the years.

During 2019/20 we provided training to some 50 producers, presenters and managers at 13 radio stations and awarded these stations a small grant to assist with production and other costs. Overall, an amount of R250 000 was earmarked for grants to stations.

The training provided by HIGHER HEALTH delivers factual content on health conditions and related social factors, and strengthens technical skills for presenting this kind of content on radio and social media platforms. Each month, HIGHER HEALTH provides the stations with content schedules that identify programme topics for the month and provide basic information that producers can interpret creatively for their audiences.

In 2019, the 13 radio stations comprising Future Beats had nearly 800 000 listeners on campus and in nearby communities.

Future Beats has been recognised by other organisations as an effective channel for reaching students and we receive

occasional funding to incorporate specific content into Future Beats programming.

- In 2019 Johnson & Johnson renewed its sponsorship of four radio stations (UNISA Radio based in Pretoria, VOW at Wits University in Johannesburg, Radio DUT at Durban University of Technology, and Maputaland Community Radio Jozini, KwaZulu-Natal) that formed part of its DREAMS Thina Abantu Abasha initiative (DREAMS TAA) which aims to empower young women. The funded radio stations produced 14 episodes following the DREAMS TAA content schedule. The partnership will continue in 2020 with increased funding.
- Wits Reproductive Health and HIV Institute provided supplementary funding on behalf of the Department of Health for the broadcast of programmes on PrEP on selected Future Beats radio stations in 2019.

The strength of the Future Beats initiative is that it is youth-driven and features diverse voices, interpreting core content in very different ways that are relevant to each station's unique audience. The resulting products can be outstanding: VOW fm's show, Love Shack (which utilises Future Beats content), was nominated in the Best Actuality Show category of the 2019 South African Radio Awards.

Table 15: Future Beats radio partners 2019 and 2020

Radio station by province	Listeners
Eastern Cape	
Alfred Nzo (Community)	138 000
Forte FM (Campus)	7 000
Madibaz FM (Campus)	15 000
Mdantsane FM (Community)	61 000
Rhodes Music Radio (Campus)	21 800
Unitra FM (Community)	240 000
Free State	
CUT FM (Campus)	38 000
Kovsie FM (Campus)	28 000
Gauteng	
Tshwane FM (Campus)	27 500
UNISA Radio (Campus)	15 000
VOW FM (Campus)	20 000
VUT fm (Campus)	35 000

KwaZulu-Natal	
Durban Youth Radio (Community)	12 000
Maputaland Radio (Community)	85 000
Radio DUT (Campus)	8 000
Zululand Radio (Campus)	38 000
Limpopo	
Radio Turf (Campus)	6 000
Univen Radio (Campus)	9 000
Mpumalanga	
Barberton Radio (Community)	55 000
Emalahleni FM (Community)	97 000
North West	
Mahikeng FM (Community)	88 000
Western Cape	
UCT Radio (Campus)	1 000

Note: Green shading indicates inclusion from 2020

2.4

CORPORATE COMMUNICATIONS

HIGHER HEALTH utilises social media, the mass media and its own communication platforms to speak to the general public and our stakeholders about the work we do. This year the rebranding of the organisation – and particularly the creation of a new visual identity – took centre stage in this area of operation.

REBRANDING

We commissioned the creative agency, Breinstorm, to conceptualise a new name for our organisation – known then as HEAIDS – and to develop our new visual identity. This was a process that required considerable discussion, but ultimately consensus was reached and the process of rebranding began. This involved:

- Creating new corporate stationery and branding materials.
- Adopting a new internet domain and redesigning the website.
- Creating new social media properties.
- Developing materials with the new branding for use during campus events.

MEDIA RELATIONS

HIGHER HEALTH engages the media proactively not only to create awareness of its work but also to advocate for action on major health issues that impact on students. In recent years, we have consistently raised the reality of GBV within our campus communities, and more recently highlighted the extent to which students experience mental health problems. The idea of using the media as a platform for dialogue on these issues is to pave the way for more focused interventions to address the identified issues.

We were fortunate to have SABC3 Current Affairs produce a 30-minute documentary on the work of HIGHER HEALTH, showing First Things First activations and other student dialogues and interviewing a range of people, from the CEO to students.

SOCIAL MEDIA

New Twitter and LinkedIn accounts were created in September to reflect the organisation's new name and look. Our LinkedIn presence has more of a corporate feel and our aim is to refresh it weekly with new posts. The Twitter account is more dynamic and requires daily posts.

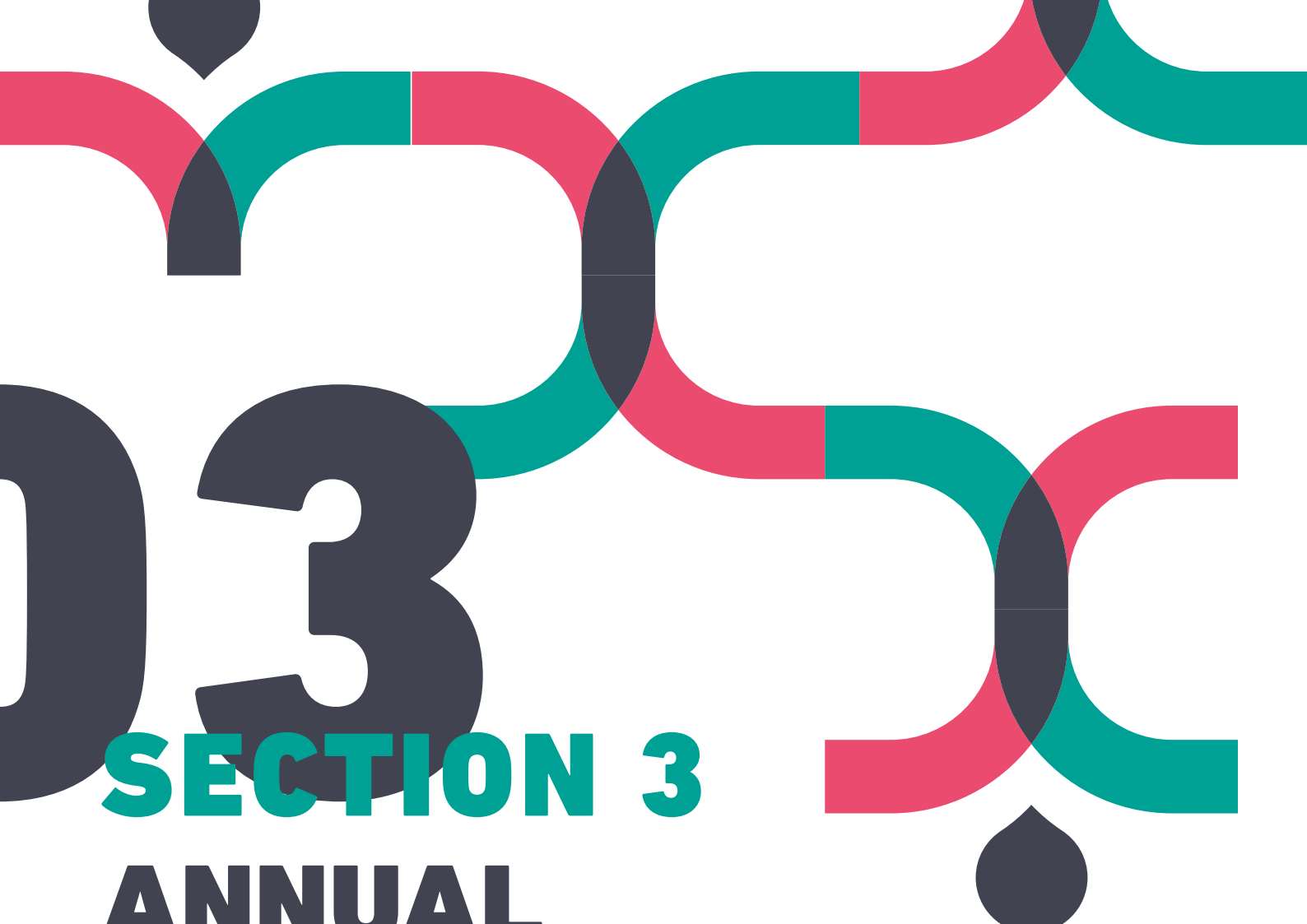
Generally, HIGHER HEALTH does not pay to boost its social media visibility. However, we did so for a limited period between September and December 2019 to establish our LinkedIn presence. The number of monthly impressions grew rapidly to over 20 000 by November but declined equally sharply to around 2 000 a month once our sponsorship ended. The organic growth of social media, driven by audience interest, is a slow process with a relatively low ceiling. However, LinkedIn remains a useful corporate “shop window” and a way of connecting regularly with individuals who have a sustained interest in our work.

STAKEHOLDER COMMUNICATION

The HIGHER HEALTH website is an important tool for connecting with stakeholders. When redesigning the website to reflect the new visual identity we also took care to ensure:

- The architecture of the site facilitates the display of information on all our focus areas and programme activities. When fully populated, it will be an information-rich platform.
- The principle of “mobile first” design was adopted, so that students who access digital platforms mainly from mobile devices would be comfortable on the site.

The CEO's very full programme of engagements ensures that there is regular face-to-face contact with numerous important stakeholders in the higher education sector, the public health field and the development community more broadly.



03

SECTION 3

ANNUAL FINANCIAL STATEMENTS FOR THE YEAR ENDED 31 MARCH 2020

**Higher Education and
Training Health, Wellness and
Development Centre NPC**

GENERAL INFORMATION

Country of incorporation and domicile	South Africa
Registration number	2017/419731/08
Nature of business and principal activities	Health and wellness services and capacity development in higher education institutions, including universities and TVET colleges
Directors	Prof Ahmed Cassim Bawa Dr Ramneek Ahluwalia Mr Samuel Zamokuhle Zungu Prof Nancy Metse Phaswana-Mafuya Prof Tokozile Valerie Mayekiso Mrs Hellen Manakedi Ntlatleng Dr Doeke Tromp Mr Mahlubi Mabizela Mrs Ashwanthee Singh Prof Willem de Villiers Mr Brian Madalane (Appointed 5 Dec 2019) Mr Sanele Mlotshwa (Appointed 5 Dec 2019)
Business address	Wild Olive Building, 262 Rose Avenue, Riverfalls Office Park, Centurion, Pretoria 0157
Postal address	Wild Olive Building, 262 Rose Avenue, Riverfalls Office Park, Centurion, Pretoria 0132
Bank	ABSA Bank Ltd
Level of assurance	These financial statements have been audited in compliance with the applicable requirements of the Companies Act 71 of 2008 South Africa
Auditor	PricewaterhouseCoopers Inc Chartered accountants (SA) Registered auditors
Preparer	The annual financial statements were internally compiled by Macdonald Takundwa

DIRECTORS' RESPONSIBILITIES AND APPROVAL

The directors are required by the Companies Act 71 of 2008 to maintain adequate accounting records and are responsible for the content and integrity of the annual financial statements and related financial information included in this report. It is their responsibility to ensure that the annual financial statements satisfy the financial reporting standards with regard to form and content and present fairly the statement of financial position, results of operations and business of the company, and explain the transactions and financial position of the business of the company at the end of the financial year. The annual financial statements are based on appropriate accounting policies, consistently applied throughout the company, and supported by reasonable and prudent judgments and estimates.

The directors acknowledge that they are ultimately responsible for the system of internal financial control established by the company and place considerable importance on maintaining a strong control environment. To enable the directors to meet these responsibilities, the Board of Directors sets standards for internal control aimed at reducing the risk of error or loss in a cost-effective manner. The standards include the proper delegation of responsibilities within a clearly defined framework, effective accounting procedures and adequate segregation of duties to ensure an acceptable level of risk. These controls are monitored throughout the company and all employees are required to maintain the highest ethical standards in ensuring the company's business is conducted in a manner that, in all reasonable circumstances, is above reproach.

The focus of risk management in the company is on identifying, assessing, managing and monitoring all known forms of risk across the company. While operating risk cannot be fully eliminated, the company endeavours

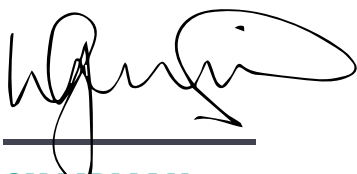
to minimise it by ensuring that appropriate infrastructure, controls, systems and ethical behaviour are applied and managed within predetermined procedures and constraints.

The directors are of the opinion, based on the information and explanations given by management and the external auditor, that the system of internal control provides reasonable assurance that the financial records may be relied on for the preparation of the annual financial statements. However, any system of internal financial control can provide only reasonable, and not absolute, assurance against material misstatement or loss.

The going-concern basis has been adopted in preparing the financial statements. Based on the forecasts and available cash resources, the directors have no reason to believe that the company will not be a going concern in the foreseeable future. The financial statements support the viability of the company.

The annual financial statements have been audited by the independent auditing firm, PricewaterhouseCoopers Inc, who have been given unrestricted access to all financial records and related data, including minutes of all meetings of the member, the Board of Directors, and committees of the Board of Directors. The Board of Directors believes that all representations made to the independent auditor during the audit were valid and appropriate. The external auditor's unqualified audit report is presented on pages 45 to 47.

The annual financial statements, set out on pages 48 to 50, and the supplementary information set out on page 40 – which have been prepared on a going-concern basis – were approved by the Board of Directors and were signed on 31 July 2020 on their behalf by:



CHAIRMAN



CHIEF EXECUTIVE OFFICER

REPORT OF THE AUDIT & RISK AND FINANCE & INVESTMENT COMMITTEE

We are pleased to present our report for the financial year ended 31 March 2020.

The Audit & Risk and Finance & Investment Committee is an independent statutory committee appointed by the directors of the company. The committee has conducted its affairs in compliance with its terms of reference.

1. AUDIT & RISK AND FINANCE & INVESTMENT COMMITTEE MEMBERS

The Audit & Risk and Finance & Investment Committee is independent and consists of five independent, non-executive directors. It meets at least twice a year as per its terms of reference.

The Chairperson of the Board, Chief Executive Officer, Finance Director and External Auditor and other assurance providers (legal, compliance, risk, health and safety) attend meetings by invitation only.

Members of the committee are:

Name of member	Position	Appointed	Institution
Mr Theo Madurai	Chairperson	21/11/2017	Tshwane University of Technology Director: Financial Control
Prof Tokozile Mayekiso		21/11/2017	University of Mpumalanga Vice-Chancellor
Dr Doeke Tromp		21/11/2017	Independent Former Chief Financial Officer of Tshwane University of Technology
Mr Eminos Manyawi		15/3/2019	Sefako Makgatho Health Sciences University Chief Financial Officer
Mr Brian Madalane		15/3/2019	Northern Cape TVET College Principal

2. MEETINGS HELD BY THE AUDIT & RISK AND FINANCE & INVESTMENT COMMITTEE

The Audit & Risk and Finance & Investment Committee performs the duties laid upon it by section 94(7) of the Companies Act 71 of 2008 by holding meetings with key role players on a regular basis and by the unrestricted access granted to the external auditor.

which includes consideration of previous appointments of the auditor, the extent of other work undertaken by the auditor for the company, and compliance with criteria relating to independence and conflict of interests, as prescribed by the Independent Board for Auditors. Requisite assurance was sought and provided by the auditor that internal governance processes within the audit firm support and demonstrate its claim to independence.

3. EXTERNAL AUDITOR

The audit committee has satisfied itself that the external auditor was independent of the company, as set out in section 94(8) of the Companies Act 71 of 2008 South Africa,

The committee ensured that the appointment of the auditor complied with the Companies Act 71 of 2008 and other legislation relating to the appointment of auditors. The committee, in consultation with executive management, agreed to the engagement letter, terms, audit plan and budgeted audit fees for the 2020 year.

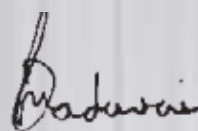
4. ANNUAL FINANCIAL STATEMENTS

The audit committee has reviewed the accounting policies and the financial statements of the company and is satisfied that they are appropriate and comply with International Financial Reporting Standards.

5. EFFECTIVENESS OF INTERNAL CONTROL

The system of internal control applied by the entity over financial reporting and risk management is effective, efficient and transparent. From the audit report on the financial statements and the management report of the external auditor, it was noted that no matters were reported

that indicate any material deficiencies in the system of internal control or any deviations therefrom. Accordingly, the committee can report that the system of internal control over financial reporting for the period under review was effective and efficient.



THEO MADURAI
CHAIRPERSON OF THE AUDIT & RISK AND FINANCE & INVESTMENT COMMITTEE

31 July 2020

DIRECTORS' REPORT

The directors present their report for the year ended 31 March 2020

1. REVIEW OF ACTIVITIES

Main business and operations

The principal activity of the company is the development of health and wellness services and capacity in higher education institutions, which include universities and TVET colleges. There were no major changes in this respect during the year.

The company changed its trading name from HEAIDS to HIGHER HEALTH during the year.

The operating results and statement of financial position of the company are fully set out in the attached financial statements and do not, in our opinion, require any further comment.

2. GOING CONCERN

The annual financial statements have been prepared on the basis of accounting policies applicable to a going concern. This basis presumes that funds will be available to finance future operations and that the realisation of assets and settlement of liabilities, contingent obligations and commitments will occur in the ordinary course of business.

The directors have assessed the impact of the COVID-19 pandemic and are satisfied that the company's operations and cashflows will not be negatively affected. Over and above its normal funding, the company has managed to secure from its donors funding amounting to R27 million for COVID-19 initiatives.

The directors believe that the company has adequate financial resources to continue in operation for the foreseeable future and accordingly the annual financial statements have been prepared on a going-concern basis.

The directors are not aware of any other material changes that may adversely impact on the company. The directors are also not aware of any material non-compliance with statutory or regulatory requirements or of any pending changes to legislation which may affect the company.

The company incurred a net surplus for the year ended 31 March 2020 of R2 915 834 (2019 surplus: R119 109). The company continues to incur surpluses.

3. EVENTS AFTER THE REPORTING DATE

Between the end of the reporting period and the date when the financial statements were to be signed off, there was a worldwide outbreak of the COVID-19 virus that has, to date, had a significant negative impact on the operation of companies as well as the movement of people due to lockdowns imposed by governments in an effort to contain the pandemic. As the company's revenue is mainly grant income and donations, management is concerned that funding from donors and grantors may be affected as the economy contracts and new priorities may arise. The budget for the 2021 financial year is mostly secured but management will continue to monitor developments on renewal applications and funding prospects. At the time of reporting, South Africa is in the early stages of the outbreak and there is a high level of uncertainty as to prevention and containment of the pandemic and the possible impact on the economy. This makes reasonable impact assessments difficult at this point in time, but it is considered that the financial impact will be insignificant to the company as most grants are committed for at least until 2022.

All events subsequent to the date of the annual financial statements, and for which the applicable financial reporting framework requires adjustment or disclosure, have been adjusted or disclosed.

4. DIRECTORS' INTEREST IN CONTRACTS

To our knowledge none of the directors had any interest in contracts entered into during the year under review.

5. DIRECTORS

Prof Ahmed Cassim Bawa
Dr Ramneek Ahluwalia
Mr Samuel Zamokuhle Zungu
Prof Nancy Metse Phaswana-Mafuya
Prof Tokozile Valerie Mayekiso
Mrs Hellen Manakedi Ntlatleng
Dr Doeke Tromp
Mr Mahlubi Mabizela
Mrs Ashwanthee Singh
Prof Willem de Villiers
Mr Sanele Mlotshwa
Mr Brian Madalane

6. SECRETARY

No secretary has been formally appointed during the current financial year.

7. PROPERTY, PLANT AND EQUIPMENT

There were no changes in the nature of property, plant and equipment of the company or in policy regarding their use.

8. INDEPENDENT AUDITOR

PricewaterhouseCoopers Inc was the independent auditor for the year under review.



Independent auditor's report

To the Members of Higher Education Training Health Wellness Development Centre NPC

Our opinion

In our opinion, the financial statements present fairly, in all material respects, the financial position of Higher Education Training Health Wellness Development Centre NPC (the Company) as at 31 March 2020, and its financial performance and its cash flows for the year then ended in accordance with the International Financial Reporting Standards and the requirements of the Companies Act of South Africa.

What we have audited

Higher Education Training Health Wellness Development Centre NPC's financial statements set out on pages 48 to 50 comprise:

- the statement of financial position as at 31 March 2020;
- the statement of surplus or deficit and other comprehensive income for the year then ended;
- the statement of changes in equity for the year then ended;
- the statement of cash flows for the year then ended;
- accounting policies; and
- the notes to the annual financial statements.

Basis for opinion

We conducted our audit in accordance with International Standards on Auditing (ISAs). Our responsibilities under those standards are further described in the *Auditor's responsibilities for the audit of the financial statements* section of our report.

We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion.

Independence

We are independent of the Group in accordance with the sections 290 and 291 of the Independent Regulatory Board for Auditors' *Code of Professional Conduct for Registered Auditors (Revised January 2018)*, parts 1 and 3 of the Independent Regulatory Board for Auditors' *Code of Professional Conduct for Registered Auditors (Revised November 2018)* (together the IRBA Codes) and other independence requirements applicable to performing audits of financial statements in South Africa. We have fulfilled our other ethical responsibilities, as applicable, in accordance with the IRBA Codes and in accordance with other ethical requirements applicable to performing audits in South Africa. The IRBA Codes are consistent with the corresponding sections of the International Ethics Standards Board for Accountants' *Code of Ethics for Professional Accountants* and the International Ethics Standards Board for Accountants' *International Code of Ethics for Professional Accountants (including International Independence Standards)* respectively.

PricewaterhouseCoopers Inc., 4 Lisbon Lane, Waterfall City, Jukskei View, 2090
Private Bag X36, Sunninghill, 2157, South Africa
T: +27 (0) 11 797 4000, F: +27 (0) 11 209 5800, www.pwc.co.za

Chief Executive Officer: L S Machaba

The Company's principal place of business is at 4 Lisbon Lane, Waterfall City, Jukskei View, where a list of directors' names is available for inspection.
Reg. no. 1998/012055/21, VAT reg.no. 4950174682.



Other information

The directors are responsible for the other information. The other information comprises the information included in the document titled “Higher Education Training Health Wellness Development Centre NPC Annual Financial Statements for the year ended 31 March 2020” which includes the Directors’ Report, as required by the Companies Act of South Africa. The other information does not include the financial statements and our auditor’s report thereon.

Our opinion on the financial statements does not cover the other information and we do not express an audit opinion or any form of assurance conclusion thereon.

In connection with our audit of the financial statements, our responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the financial statements or our knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work we have performed, we conclude that there is a material misstatement of this other information, we are required to report that fact. We have nothing to report in this regard.

Responsibilities of the directors for the financial statements

The directors are responsible for the preparation and fair presentation of the financial statements in accordance with the International Financial Reporting Standards and the requirements of the Companies Act of South Africa, and for such internal control as the directors determine is necessary to enable the preparation of financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, the directors are responsible for assessing the Company’s ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless the directors either intend to liquidate the Company or to cease operations, or have no realistic alternative but to do so.

Auditor’s responsibilities for the audit of the financial statements

Our objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor’s report that includes our opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with ISAs will always detect a material misstatement when it exists. Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these financial statements.

As part of an audit in accordance with ISAs, we exercise professional judgement and maintain professional scepticism throughout the audit. We also:

- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for our opinion. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Company’s internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by the directors.
- Conclude on the appropriateness of the directors’ use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events



or conditions that may cast significant doubt on the Company's ability to continue as a going concern. If we conclude that a material uncertainty exists, we are required to draw attention in our auditor's report to the related disclosures in the financial statements or, if such disclosures are inadequate, to modify our opinion. Our conclusions are based on the audit evidence obtained up to the date of our auditor's report. However, future events or conditions may cause the Company to cease to continue as a going concern.

- Evaluate the overall presentation, structure and content of the financial statements, including the disclosures, and whether the financial statements represent the underlying transactions and events in a manner that achieves fair presentation.

We communicate with the directors regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that we identify during our audit.

A handwritten signature in black ink that reads "PricewaterhouseCoopers Inc." in a cursive script.

PricewaterhouseCoopers Inc.

Director: Raj Dhanlall

Registered Auditor

Waterfall City, Johannesburg

9 September 2020

STATEMENT OF FINANCIAL POSITION

Figures in R

	Notes	2020	2019
Non-current assets			
Property, plant and equipment	4	3 846 044	715 734
Total non-current assets		3 846 044	715 734
Current assets			
Trade and other receivables	5	831 816	1 491 210
Cash and cash equivalents	6	29 479 356	20 488 618
Total current assets		30 311 172	21 979 828
Total assets		34 157 216	22 695 562
Equity and liabilities			
Equity			
Accumulated surplus		3 739 619	823 785
Liabilities			
Current liabilities			
Provisions	7	450 766	719 551
Trade and other payables	8	2 828 688	1 907 342
Deferred income	9	27 138 143	19 244 884
Total current liabilities		30 417 597	21 871 777
Total liabilities		30 417 597	21 871 777
Total equity and liabilities		34 157 216	22 695 562



STATEMENT OF SURPLUS OR DEFICIT AND OTHER COMPREHENSIVE INCOME

Figures in R

	Notes	2020	2019
Revenue	10	45 251 799	33 817 106
Administrative expenses	11	(1 788 189)	(1 321 769)
Other expenses		(40 420 033)	(32 376 228)
Surplus from operating activities		3 043 577	119 109
Finance costs		(127 743)	-
Surplus for the year		2 915 834	119 109

STATEMENT OF CHANGES IN EQUITY

Figures in R

	Accumulated surplus	Total
Balance at 1 April 2018	704 676	704 676
Changes in equity		
Surplus for the year	119 109	119 109
Total comprehensive income	119 109	119 109
Balance at 31 March 2019	823 785	823 785
Balance at 1 April 2019	823 785	823 785
Changes in equity		
Surplus for the year	2 915 834	2 915 834
Total comprehensive income	2 915 834	2 915 834
Balance at 31 March 2020	3 739 619	3 739 619





HIGHER HEALTH

Higher Education & Training: Health, Wellness and Development Centre

ANNUAL REPORT

2019
2020

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